

PLACEHOLDER

EVERGREEN COMMUNITY CARE INC.

RESIDENTIAL CARE PROGRAM MANUAL

Standards for Staffed Children's Residential Services

Aligned to MCFD Standards (Revised April 2026) — CFCSA — CCALA — RCR — DRIPA

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Secondary Legislation	Declaration on the Rights of Indigenous Peoples Act (DRIPA); BC Human Rights Code; FOIPPA; Workers Compensation Act; Mental Health Act; Criminal Records Review Act; Motor Vehicle Act
MCFD Standards	Standards for Staffed Children's Residential Services (MCFD, revised April 2026) — Standards A through H
Licensing Body	BC Ministry of Health / Health Authority Licensing Office
Contract Authority	BC Ministry of Children and Family Development (MCFD)
Applies To	All staff, volunteers, contractors, and practicum students; all children and youth placed by MCFD in Evergreen Community Care residential programs
Manual Sections	30 Sections + 12 Appendices — covering all MCFD Standards A–H and all operational practice areas

DOCUMENT INFORMATION

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PART I — LEGISLATIVE AND ORGANIZATIONAL FRAMEWORK

1. LEGISLATIVE AND REGULATORY FRAMEWORK

Regulatory Reference: CFCSA s.2–4; CCALA; Residential Care Regulation; DRIPA; BC Human Rights Code

1.1 Primary Legislation

Evergreen Community Care's residential care services are delivered within a comprehensive BC legislative framework. Every policy, procedure, and practice described in this Manual must be understood and applied within this framework.

Legislation / Regulation	Application to Residential Care
Child, Family and Community Service Act (CFCSA, 1996)	Governs all child welfare services in BC; establishes Guiding Principles, Service Delivery Principles, Best Interests of the Child, Rights of Children in Care (s.70), mandatory duty to report (s.14), and placement decisions
Child in Care Regulation	Sets specific rights and entitlements for children in government care; governs allowances, personal property, privacy, and contact
Community Care and Assisted Living Act (CCALA)	Governs licensing of community care facilities; requires a valid licence to operate; empowers Licensing Officers
Residential Care Regulation (BC Reg. 96/2009, last amended April 1, 2026)	Sets detailed physical, operational, and staffing standards for licensed residential care facilities; governs admission, records, staffing ratios, and physical environment
Declaration on the Rights of Indigenous Peoples Act (DRIPA, 2019)	Implements UNDRIP in BC; requires alignment of provincial laws and policies; informs all Indigenous child placement and service decisions
BC Human Rights Code (RSBC 1996, c. 210)	Prohibits discrimination; protects gender identity, sexual orientation, disability, race, and all other protected grounds for youth and staff
Freedom of Information and Protection of Privacy Act (FOIPPA)	Governs collection, use, disclosure, and protection of personal information in individual service records and organizational files
Criminal Records Review Act (RSBC 1996, c. 86)	Requires criminal record checks for all staff, volunteers, and contractors with access to children
Workers Compensation Act / OHS Regulation	Governs workplace health and safety — see OHS Program Binder (ECC-OHS-POL-2026-002)
Mental Health Act (RSBC 1996, c. 288)	Governs involuntary psychiatric detention; informs staff response to youth in acute psychiatric crisis
Motor Vehicle Act	Governs vehicle safety, driver licensing, and child restraint requirements for youth transport

1.2 CFCSA Guiding Principles (s.2)

The following Guiding Principles from section 2 of the CFCSA form the foundation of every practice and decision at Evergreen Community Care. Every worker must know and uphold these principles:

- The safety and well-being of children are the paramount considerations
- Children are entitled to be protected from abuse, neglect, and harm or threat of harm
- A family is the preferred environment for the care and upbringing of children and the responsibility for the protection of children rests primarily with the parents
- Indigenous families and Indigenous communities share responsibility for the upbringing and well-being of Indigenous children
- If, with available support services, a family can provide a safe and nurturing environment for a child, support services should be provided
- The child's views should be taken into account when decisions relating to a child are made
- Kinship ties and a child's attachment to the extended family should be preserved if possible
- The cultural identity of Indigenous children should be preserved
- Indigenous children are entitled to learn about and practise their Indigenous traditions, customs, and languages, and to belong to their Indigenous communities
- Decisions relating to children should be made and implemented in a timely manner

1.3 CFCSA Service Delivery Principles (s.3)

- Families and children should be informed of services available to them and encouraged to participate in decisions that affect them
- Indigenous people should be involved in the planning and delivery of services to Indigenous families and their children
- Services should be planned and provided in ways sensitive to the needs and cultural, racial, and religious heritage of those receiving services
- The impact of residential schools on Indigenous children, families, and communities should be considered in the planning and delivery of services
- Services should be integrated, wherever possible, with services provided by other government ministries, community agencies, and CLBC
- The community should be involved, wherever possible, in the planning and delivery of services

1.4 Best Interests of the Child

All actions and decisions at Evergreen Community Care must consider the best interests of each child. The CFCSA requires consideration of all relevant factors, including:

Factor	Application at Evergreen Community Care
Child's safety	No decision is made that increases risk to a child's physical or psychological safety
Physical and emotional needs	Care plans are individualized and responsive to each child's developmental stage and needs
Continuity of care	Placement stability is prioritized; unnecessary moves are avoided
Relationship quality and continuity	Family contact is supported unless a court order or safety plan prohibits it
Cultural, racial, linguistic, and religious heritage	Cultural plans are developed for all children, with specific requirements for Indigenous youth

Child's own views	Children are meaningfully consulted on decisions affecting them, in an age-appropriate way
Timeliness	Decision-making is not delayed beyond what is necessary to gather information
For Indigenous children: cultural connection	Additional factors include the child's connection to their Nation, community, language, and traditional practices

2. RIGHTS OF CHILDREN AND YOUTH IN CARE

Regulatory Reference: CFCSA s.70; Child in Care Regulation; MCFD Standards B.1–B.5

2.1 Legal Foundation — CFCSA Section 70

The rights of children and youth in care are enshrined in section 70 of the CFCSA as legal entitlements that are enforceable and non-waivable. Every staff member must know these rights and ensure that the care they provide is consistent with them. Rights are not contingent on behaviour; they cannot be withdrawn as a consequence.

⚠️ A child's rights under s.70 CFCSA cannot be used as a reward or removed as a punishment. Any practice that treats a legal right as a privilege is a violation of the CFCSA and MCFD Standards.

2.2 Rights of Every Child and Youth in Care (CFCSA s.70)

Every child and youth in the care of Evergreen Community Care has the following rights:

- To be fed, clothed, and nurtured according to community standards, and to be given the same quality of care as other children in the placement, including the caregiver's own children
- To be informed about their plans of care in a manner appropriate to their age and cognitive ability
- To be consulted and to express their views, according to their abilities, about significant decisions affecting them
- To reasonable privacy and possession of their personal belongings
- To be free from corporal punishment
- To be informed of the standard of behaviour expected by their caregivers and of the consequences of not meeting those expectations
- To receive medical and dental care when required
- To participate in social and recreational activities if available and appropriate, and according to their abilities and interests
- To receive the religious instruction and to participate in the religious activities of their choice
- To receive guidance and encouragement to maintain their cultural heritage
- To be provided with an interpreter if language or disability is a barrier to consulting with them on decisions affecting their custody or care
- To privacy during discussions with members of their families, subject to any court order

2.3 Additional Rights of Indigenous Children and Youth

In addition to the rights in section 70 of the CFCSA, Indigenous children and youth placed at Evergreen Community Care are entitled to:

- Be placed with Indigenous family members, extended family, a family approved by their Nation, or another Indigenous family, in that order of preference (CFCSA s.3(c))
- Maintain connection with their Nation, community, language, culture, traditions, and ceremonies
- Have a Cultural Plan developed within 30 days of placement in consultation with the child, their family, and their Nation
- Access Elders, cultural mentors, Delegated Aboriginal Agency (DAA) representatives, and traditional healing practices
- Have their cultural and linguistic identity affirmed and supported in their daily care

- Have the impact of residential schools and colonization acknowledged and considered in all service planning

2.4 Informing Youth of Their Rights

Every child and youth is informed of their rights in an age-appropriate, accessible manner within 48 hours of placement:

- Staff review the Rights Information Handout (Appendix C) with the youth orally during orientation
- The handout is provided to the youth in plain language; alternate formats (visual, audio, translated) are arranged if needed
- The youth's understanding of their rights is confirmed and documented in the admission record
- Rights information is posted in a visible location in the common area of the facility
- Youth are informed of their right to complain, including how to access the MCFD Advocate for Children and Youth: 1-800-476-3933

2.5 Rights Monitoring

Program Managers review rights compliance as part of the monthly program quality review (Appendix J). Any concern that a youth's rights may have been violated is treated as a critical incident and reported immediately to the Executive Director and the youth's MCFD social worker. Evergreen Community Care does not investigate rights violation allegations internally without MCFD involvement.

3. BEST INTERESTS OF THE CHILD DECISION-MAKING FRAMEWORK

Regulatory Reference: CFCSA s.4; MCFD Child Welfare Policy Chapter 2

3.1 Purpose

Every significant decision affecting a child or youth in Evergreen Community Care's care must be made in the child's best interests, as defined by the CFCSA and MCFD policy. This section provides a practical framework for applying the best interests standard in day-to-day and significant decisions.

3.2 Decision-Making Framework

Decision Type	Best Interests Process	Documentation
Day-to-day care decisions (meals, activities, routines)	Apply the 'reasonable and prudent parent' standard; consider the child's preferences, age, and wellbeing	Shift log; mention in monthly ISP notes
Health care decisions	Consult the child's MCFD social worker for non-emergency decisions; 911 + notify MCFD for emergencies	Health care log; incident report if urgent
Contact with family	Follow the contact schedule in the care plan; seek MCFD guidance for changes	Shift log; ISP update
Educational decisions	Consult social worker; support school attendance and engagement as the default	ISP; education support log
Community activities	Assess risk, developmental appropriateness, and the child's interest; use the outing safety plan template	Shift log; Appendix K for community outings
Significant purchases or financial decisions	Follow the Finance Manual (ECC-FA-2026-001) and MCFD allowance guidelines	Finance record; ISP note
Placement changes	Initiated by MCFD; Evergreen Community Care provides input through the care team	ISP; transition planning checklist
Cultural and religious activities	Default to supporting participation; consult social worker if concerns about specific activities	ISP; cultural plan update

3.3 Balancing Safety and Reasonable Risk

Children and youth in care have the right to age-appropriate activities that carry ordinary risk, just as children in family care do. Evergreen Community Care does not restrict youth activities beyond what a reasonable, prudent parent would. Excessive restriction of activities for the sole purpose of avoiding any organizational risk is contrary to the best interests standard and the CFCSA.

When assessing whether to permit an activity, staff ask: 'Would a reasonable, prudent parent allow this for a child of this age and ability?' If yes, the activity is permitted. If the child's individual safety plan or care plan specifically restricts an activity, that restriction is followed.

4. ORGANIZATIONAL MISSION, VALUES, AND PROGRAM PHILOSOPHY

4.1 Mission Statement

Evergreen Community Care Inc. provides high-quality, trauma-informed, rights-based residential care to children and youth in the care of MCFD, supporting their safety, healing, growth, and successful transition to independence or family.

4.2 Vision

Every child and youth in our care experiences a stable, nurturing home environment in which they feel safe, respected, and supported to reach their full potential.

4.3 Core Values

Value	What This Means in Practice
Safety	Physical and psychological safety is the non-negotiable foundation of every shift, every decision, and every interaction
Dignity and Respect	Every youth is treated with dignity, irrespective of their behaviour; we see the person, not just the presenting challenge
Trauma-Informed Care	We understand that behaviour is communication; we respond to the underlying need, not just the surface behaviour
Cultural Safety and Humility	We actively create environments where Indigenous and racialized youth feel culturally safe and affirmed
Rights-Based Practice	We know and uphold the rights of every child in our care; rights are never used as rewards or consequences
Transparency and Accountability	We are honest with youth, families, MCFD, and each other; we own our mistakes and learn from them
Family and Community	We support youth to maintain meaningful connections with family, community, and culture wherever it is safe and in their best interests

4.4 Program Philosophy

Evergreen Community Care operates its residential programs on the following foundational principles:

- Residential care is a therapeutic environment, not a punitive one. Youth in our care have experienced harm and loss. Our role is to provide stability, healing, and hope.
- Relationships are the primary vehicle of healing. Consistent, caring relationships with staff are more therapeutic than any program, intervention, or technique.
- Every youth can grow. We hold high expectations for every youth in our care and support them to meet those expectations through encouragement, skill-building, and graduated responsibility.
- Cultural safety is not optional. For Indigenous youth especially, cultural connection is a protective factor and a right. We actively facilitate cultural connection, not just accommodate it.

- Families are partners. Wherever it is safe, we involve family in the youth's care, support family contact, and work toward family reunification or other permanency.
- Staff are the program. The quality of care is determined by the quality of the people delivering it. We invest in our staff's wellbeing, training, and professional growth.

5. CODE OF ETHICS AND PROFESSIONAL CONDUCT

Regulatory Reference: CFCSA s.14; MCFD Standards G.4; BC Human Rights Code; Criminal Records Review Act

5.1 Application

This Code of Ethics applies to all persons working at or for Evergreen Community Care, including employees, contractors, practicum students, and volunteers. Compliance with this Code is a condition of continued engagement. Violations of this Code are investigated and addressed under the HR Policy Binder (ECC-HR-POL-2026-001).

5.2 Foundational Ethical Obligations

Obligation	Detail
Prioritize the child's best interests	In every interaction, the safety and wellbeing of the child or youth comes first — before organizational interests, staff preferences, or family wishes
Mandatory duty to report	Every staff member has an absolute and personal duty to report any information that leads them to reasonably believe a child needs protection, per CFCSA s.14. This duty cannot be delegated. It is not discharged by telling a supervisor — the duty to report to MCFD or police is personal.
Confidentiality	Information about youth and families is collected only for the purpose of providing care. It is shared only on a need-to-know basis and never disclosed to unauthorized persons, including unauthorized family members or members of the public.
Criminal Record Check compliance	Every staff member, volunteer, and contractor with access to children holds a current, satisfactory Criminal Record Check (CRC) Vulnerable Sector clearance before beginning work or volunteering.
Professional boundaries	Staff maintain clear professional boundaries with youth at all times. Dual relationships (personal friendships, romantic relationships, financial relationships) between staff and current youth or their families are prohibited.
Non-violence and restraint limitation	Physical restraint is used only in situations of imminent danger to life, using only trained techniques, for the minimum time necessary, and is always reported and reviewed. Corporal punishment is prohibited.
Honesty and transparency	Staff are honest with youth, families, MCFD, supervisors, and colleagues. Documentation is accurate and contemporaneous. Falsifying records is grounds for termination.
No discrimination	No youth or family receives lesser care or service on the basis of race, culture, religion, gender identity, sexual orientation, disability, family structure, or any other ground protected under the BC Human Rights Code.
Gifts and personal benefit	Staff do not accept gifts of significant value from youth or families; do not borrow money from youth or families; and do not benefit personally from any youth-related financial transaction.
Social media	Youth and families are never discussed on any social media platform. Photographs of youth are never taken without written authorization per the

Media Consent and Image Policy.

5.3 Mandatory Reporting Obligation — CFCSA s.14

⚠ This is the most important obligation in this section. Read it carefully. Every employee, contractor, and volunteer has a personal, non-delegable duty to report.

Under CFCSA section 14, a person who has reasonable grounds to believe that a child:

- Has been or is likely to be physically harmed, sexually abused or exploited, or tortured or abandoned by a parent or carer
- Needs protection because a person caring for the child is unable or unwilling to protect the child
- Has been or is likely to be harmed because the child's parent or carer is unable to care adequately for the child
- Is in the care of, or has been harmed by, a person other than a parent

...must promptly report the matter to a Director (MCFD child protection worker). This duty applies:

- Even if the information is confidential, or the concern was shared in confidence
- Even if you reported a previous incident involving the same child
- Even if you believe someone else has already reported
- Even if the concern involves a colleague or supervisor
- To every person separately — it cannot be delegated to a supervisor to report on your behalf

Failure to report is an offence under the CFCSA and may also give rise to professional disciplinary action. Reporting in good faith is protected even if the report is not substantiated.

MCFD Child Protection Line: 1-800-663-9122 (24 hours / Provincial Centralized Screening)

5.4 Professional Boundaries

Healthy therapeutic relationships require clear professional boundaries. The following boundaries apply to all staff:

Boundary	Required Practice
Physical contact	Physical contact with youth is guided by the youth's comfort, age, and care plan; never for staff gratification; never sexual in any way
Self-disclosure	Staff share personal information with youth only when therapeutically appropriate and with supervisor awareness; avoid burdening youth with personal problems
Outside contact	Staff do not maintain contact with current or recently discharged youth on personal social media, phone, or in person without explicit supervisor and family approval
Financial relationships	Staff do not lend, borrow, or give money to youth or families; do not purchase personal items for youth outside approved budgets
Dual relationships	Staff who have pre-existing personal relationships with a youth or family member disclose this immediately to their supervisor; reassignment is considered
Favouritism	Staff provide equitable care to all youth in the home; visible favouritism is harmful to group dynamics and the non-favoured youth

PART II — MCFD STANDARDS A THROUGH H

STANDARD A — PURPOSE AND FUNCTION OF THE PROGRAM

Regulatory Reference: MCFD Standards for Staffed Children's Residential Services — Standard A; CCALA s.7; Residential Care Regulation s.7

A.1 Program Description and Purpose

Evergreen Community Care Inc. provides licensed group home residential care services for children and youth placed by MCFD in the care of the Director under the Child, Family and Community Service Act. The program provides a safe, structured, therapeutic home environment for youth aged 6–18 who cannot safely reside with their family of origin and for whom a foster home or other family-based placement is not currently available or appropriate.

A.2 Program Objectives

- Provide a safe, stable, and nurturing residential environment that meets or exceeds community standards of parenting
- Support the physical, emotional, social, cultural, and educational development of each youth in placement
- Implement individualized care and support consistent with each youth's Individual Service Plan
- Maintain meaningful connections between youth and their families, communities, and cultures
- Support each youth's progression toward the permanency goal identified in their plan of care
- Provide trauma-informed, rights-based care that acknowledges and responds to the impact of adverse childhood experiences
- Operate in full compliance with all MCFD Standards, licensing requirements, and applicable legislation

A.3 Program Type and Licensing

Program Element	Detail
Program type	Licensed group home residential care facility
Licensing authority	BC Ministry of Health / Health Authority Licensing Office (Community Care and Assisted Living Act)
Licence conditions	Current licence posted prominently at each facility; conditions reviewed and complied with at all times
Capacity	As specified in the current licence; may not be exceeded without written Licensing Officer approval
Age range served	Children and youth aged 6–18 as placed by MCFD; specific age range per licence conditions
MCFD contract	Operates under a current, valid Service Agreement with MCFD
Facility address(es)	As specified in the current licence

A.4 Admission Criteria

Evergreen Community Care accepts children and youth on the following basis:

- Placement is made by or with the approval of MCFD or the relevant Delegated Aboriginal Agency (DAA)
- The youth's care needs are within the program's capacity and competency to meet safely
- Placement does not compromise the safety of other youth currently in the home
- The facility has current licence capacity for the placement
- The Program Manager has reviewed the available information and confirmed the placement can proceed

A.5 Program Limitations and Referral

Evergreen Community Care does not accept placements that exceed the program's capacity, including:

- Youth whose documented behaviour history or current psychiatric acuity requires a more intensive residential or hospital-level treatment setting
- Youth who require medical nursing care beyond the capacity of trained residential care staff
- Placements that would exceed the licensed capacity of the facility

When a referral cannot be accepted, the Program Manager notifies the referring MCFD worker in writing within 24 hours, providing a clear explanation and, where possible, information about alternative resources.

STANDARD B — RIGHTS, COMPLAINT RESOLUTION, AND PRIVACY

Regulatory Reference: MCFD Standards B.1–B.5; CFCSA s.70; Child in Care Regulation; Residential Care Regulation s.10; FOIPPA

B.1 Rights of Children and Youth in Care

The rights of every child and youth in Evergreen Community Care's programs are described in full in Section 2 of this Manual and are incorporated by reference into this Standard.

B.2 Complaint Resolution Program

Every child, youth, and family member has the right to make a complaint about the care or service provided, without fear of retaliation. Evergreen Community Care's complaint process is:

Step	Process	Timeline
1. Informal resolution	Youth or family raises concern with their direct-care worker; worker attempts to resolve it at the point of care	Immediately or within 24 hours
2. Supervisor review	If unresolved, the Program Manager is notified and conducts a review; meets with the youth or family member	Within 3 business days
3. Executive Director review	If still unresolved, the Executive Director reviews the complaint and response	Within 7 business days of referral
4. External advocacy	Youth or family may contact the Advocate for Children and Youth (1-800-476-3933) or MCFD at any stage without going through the internal process first	At any time; no internal process required
5. MCFD contract manager	Formal complaints about services may be made to the MCFD Contract Manager	At any time
6. Licensing Officer	Complaints about licensing conditions may be made to the Health Authority Licensing Officer	At any time

Complaints and their resolutions are documented and retained. The Program Manager reviews complaint trends quarterly. No child, youth, or family member faces any adverse consequence for making a complaint.

B.3 Advocacy for Children and Youth

Evergreen Community Care actively supports youth in accessing independent advocacy:

- Office of the Advocate for Children and Youth: 1-800-476-3933 — provides free, confidential advocacy for children and youth in care
- Youth can contact the Advocate directly without staff knowledge or involvement
- Staff facilitate private phone calls with the Advocate; phone access is not monitored or restricted
- Information about the Advocate is included in the Rights Information Handout (Appendix C) provided to every youth at admission

B.4 Privacy and Confidentiality — FOIPPA Obligations

Personal information about children, youth, and families is collected, used, and disclosed in compliance with FOIPPA:

- Personal information is collected only for the purpose of providing care and services, and only to the extent necessary for that purpose
- Information is not shared with unauthorized persons; each youth's information is accessible only to staff with a legitimate need to know for care purposes
- Family members do not have automatic access to a youth's personal information; MCFD and legal guardianship status govern what family information may be shared
- Media inquiries regarding any youth are referred to the Executive Director immediately; no information is confirmed or denied to media without executive approval
- Individual service records are stored securely; physical files in locked filing cabinets; electronic records in password-protected, access-controlled systems

B.5 Youth Behaviour Management Disclosure

Youth are informed in a manner appropriate to their age and ability of: the standards of behaviour expected in the home; the supports and interventions that will be used to help them meet those standards; and the consequences of behaviours that pose safety risks to themselves or others. This information is provided at admission, reviewed during ISP meetings, and updated in the care plan when significant changes occur.

STANDARD C — PERSONAL SAFETY OF CHILDREN AND YOUTH

Regulatory Reference: MCFD Standards C.1–C.9; Residential Care Regulation s.57–70; OHS Regulation Part 4

C.1 Safe Environment Obligation

Evergreen Community Care is responsible for providing a safe, secure residential environment in which each youth is protected from harm. Safety is the foundational obligation that underlies all other program elements. A facility that is not safe cannot provide good care, regardless of the quality of its programming.

C.2 Physical Environment Safety Standards

Safety Element	Standard and Monitoring
Exterior security	All exterior entry doors lock automatically or are secured by staff; unauthorized persons cannot enter the facility unescorted; security arrangements reviewed annually and after any security incident
Fire safety	Smoke and CO detectors in all sleeping areas and common spaces tested monthly; fire extinguishers inspected annually; evacuation plan posted; quarterly fire drills; see OHS Program (s.27) for full fire safety requirements
Hazardous items	Knives, scissors, medications, cleaning chemicals, and other hazardous items are stored in locked locations accessible only to authorized staff; inventory maintained and checked each shift
Ligature risk	Ligature risk assessment completed for each facility and for each youth's bedroom; mitigation measures implemented per individual risk assessment; reviewed whenever a youth with self-harm history is placed
Water temperature	Hot water temperature at point of use does not exceed 49°C (120°F) to prevent scalding, per RCR amendment April 2026; all taps checked and thermostatic valves adjusted at admission of each youth
Sleeping arrangements	Each youth has their own bedroom or shares only with an assigned youth of appropriate compatibility; bedroom doors cannot be locked from the outside
Vehicle safety	All vehicles transporting youth are inspected before each trip; appropriate child restraints used; driver records current; see OHS Program (s.19)
Outdoor areas	Outdoor areas inspected daily before use; no unsecured hazards accessible to youth; fencing appropriate to population served

C.3 Supervision Standards

Youth are supervised at a level appropriate to their age, developmental level, individual safety plan, and the assessed risk at any given time. The minimum supervision standards are:

- Youth are never left unsupervised in the facility without a staff member present and aware of their whereabouts and activities

- During nighttime hours, a staff member is awake and on duty at all times; wellness checks are conducted at intervals specified in each youth's care plan (minimum every 30 minutes for most youth)
- Youth on community outings are supervised by a staff member with a ratio appropriate to the activity and the youth's individual needs
- Youth who have been assessed as high elopement risk are supervised at a 1:1 or enhanced ratio as specified in their safety plan
- Youth who are identified as at risk of self-harm have a supervision level specified in their care plan and safety plan; this level is communicated at every shift handover

C.4 Search Procedures

Evergreen Community Care may conduct searches of youth's belongings or personal areas where there are specific, articulable grounds to believe a youth has items that pose a safety risk to themselves or others. All searches are conducted in accordance with the following requirements:

- A supervisor is notified before any search is conducted
- Searches are conducted by a staff member of the same gender as the youth, with a second staff witness, unless an emergency prevents this
- Strip searches are not permitted under any circumstances
- Searches of a youth's bedroom are conducted with the youth present where possible, or with a MCFD social worker's guidance where the youth's presence is not safe or appropriate
- All searches are documented in the shift log: date, time, reason, what was searched, items found, disposition of any found items, and staff involved
- The youth's MCFD social worker is notified within 24 hours of any search that results in the discovery of contraband or dangerous items

C.5 Internet and Technology Safety

Youth access to the internet, social media, and personal devices is managed through the following safety framework:

- The facility's WiFi network for youth is filtered through a content control system to restrict access to age-inappropriate, harmful, or illegal content
- Youth's use of personal devices is governed by their individual care plan; social worker recommendations regarding device access are implemented
- Staff are alert to signs of online exploitation, cyberbullying, inappropriate relationships, or access to content associated with self-harm or radicalization
- Education about online safety, healthy digital boundaries, and reporting uncomfortable online interactions is provided as a life skills topic
- Concerns about a youth's online activity are documented and the MCFD social worker is notified within 24 hours

C.6 Physical and Sexual Abuse Prevention

Evergreen Community Care implements the following measures to protect youth from abuse in the residential care setting:

- All staff hold a current, clear Vulnerable Sector Criminal Record Check (CRC) before beginning work with youth
- Staff are never permitted to be alone with a single youth behind a closed door in circumstances that are not transparent and professionally appropriate
- Personal care (bathing, dressing) is conducted in a manner that preserves the youth's dignity and privacy and is only provided by staff with a legitimate care need for that role

- Youth are educated on body autonomy, consent, and the right to report discomfort with any adult in their life
- Any allegation of abuse or misconduct by a staff member is reported to MCFD and the police immediately; the staff member is placed on administrative leave pending investigation

C.7 Tobacco, Alcohol, and Substance Safety

Substance	Policy
Tobacco and vaping (youth)	Smoking and vaping are not permitted by youth inside the facility or on the property. Youth who use tobacco are provided cessation support. Youth are never supplied tobacco products by staff.
Tobacco and vaping (staff)	Staff smoke only in designated exterior areas, away from youth sightlines, and never in a way that models tobacco use to youth.
Alcohol	No alcohol is stored or consumed in residential care facilities. Youth are never provided access to alcohol.
Cannabis	Cannabis is not stored or consumed in facilities. Staff do not use cannabis within 12 hours before any shift.
Controlled substances (youth)	Medications are locked and administered only by authorized staff. Found substances are secured and reported to the social worker and police (if warranted) per the Critical Incident Protocol.
Found substances	Any substance found in the facility is secured by staff, documented in the shift log, and reported to the Program Manager; the social worker is notified within 24 hours; police contact at the Program Manager's discretion

STANDARD D — QUALITY OF SERVICE EXPERIENCES

Regulatory Reference: MCFD Standards D.1–D.8; Child in Care Regulation; Residential Care Regulation s.7–50

D.1 Daily Care — Meeting Basic Needs

Every youth in Evergreen Community Care's care receives, at minimum, the following daily care provisions:

Need	Standard
Nutrition	Three nutritious meals and two snacks per day; meals are home-cooked where possible; cultural food preferences and medically required diets are accommodated; youth are involved in meal planning and preparation as a life skill
Clothing	Adequate, weather-appropriate, age-appropriate, and personally fitting clothing; youth have input into their own clothing choices; cultural dress and expression are supported
Hygiene	Youth have access to personal hygiene products appropriate to their age and gender; privacy is ensured for all hygiene activities; staff assist only where specifically indicated in the care plan
Sleep	Each youth has their own bed with clean bedding; sleeping environment is comfortable and quiet; sleep schedule is developmentally appropriate
Recreation and leisure	Youth have access to recreational activities both in the home and in the community; age-appropriate equipment, games, and entertainment are available; screen time is balanced with active and social activities
Education	Youth are enrolled in and supported to attend school or an approved educational alternative; school attendance is monitored and supported; see Section 12
Health care	Youth's health care needs are identified and addressed; medical and dental appointments are made and attended; prescribed medications are administered as scheduled; see Section 9

D.2 Cultural and Religious Observance

Evergreen Community Care actively supports every youth's right to maintain their cultural heritage and religious practice:

- Youth attend religious services, ceremonies, and cultural events of their choice; staff facilitate transportation
- Cultural foods, traditional medicines, and ceremonial items are accommodated in the home
- Religious and cultural observances (prayer, fasting, ceremonies) are respected and supported without judgment
- Cultural Plans for Indigenous youth are developed within 30 days of placement and updated at each ISP review
- Staff receive annual Indigenous Cultural Safety and Humility training

D.3 Allowances and Personal Finances

Children and youth in care receive allowances in accordance with the Child in Care Regulation and MCFD policy:

Age Group	Approximate Weekly Allowance	Additional Provisions
6–11 years	As per MCFD schedule (reviewed annually)	Staff assist with money management; savings encouraged
12–14 years	As per MCFD schedule	Youth manages own money with staff guidance
15–18 years	As per MCFD schedule	Financial literacy skills built; budgeting supported
Youth 16+ transitioning to independence	Transition allowance as per MCFD agreement	Independent living skills; bank account, budgeting

Allowances are paid consistently on the same day each week. The youth receives their full allowance; it is not withheld as a consequence for behaviour. Any financial concerns about a youth (e.g., giving money away, being extorted) are documented and reported to the social worker.

D.4 Behaviour Support and Consequences

Behaviour support at Evergreen Community Care is grounded in trauma-informed, strengths-based principles. Consequences are logical, proportionate, and never punitive:

- Positive reinforcement is the primary tool for behaviour support; recognition of progress, strengths, and effort is a daily practice
- Consequences for behaviour are natural and logical; they are connected to the behaviour and are not humiliating, excessive, or traumatizing
- The following are never used as consequences: withholding meals; removing bedding or sleeping arrangements; restricting contact with family; humiliation in front of others; physical punishment
- Restriction of activities may be used in limited circumstances where the activity itself is directly related to the safety concern; it is always time-limited and reviewed with the youth
- All behaviour interventions are documented in the shift log and, where significant, on the Behaviour Support Plan
- Physical intervention (restraint) is a last resort for imminent safety — see Section 19

D.5 Gender-Affirming Care

Evergreen Community Care is required by MCFD Standards D.1.5(b), D.1.13(b), D.6(g), and C.1.1(b) (revised April 2024) to provide gender-affirming care to all youth. Full gender-affirming care requirements are described in Section 23 of this Manual.

- All staff use each youth's stated name and pronouns consistently; refusal or deliberate misuse is a Code of Ethics violation
- Gender-affirming clothing, personal care items, and physical environment arrangements are provided
- A youth's gender identity and expression information is confidential; it is not shared without the youth's consent
- All staff complete gender-affirming care training within 60 days of hire and annually thereafter

D.6 Recreational and Social Activities

Youth participation in recreational and social activities is not a privilege — it is a component of developmental health and a right under the CFCSA. Evergreen Community Care ensures:

- Each youth has the opportunity to participate in at least one extracurricular or recreational activity of their choice that is consistent with their interests and abilities
- Youth are encouraged and supported to develop social connections with peers in the community
- Participation in school sports, clubs, and activities is supported; staff attend youth's performances and events where possible
- Cultural and traditional activities are facilitated as part of the youth's Cultural Plan

D.7 Mealtime and Nutrition

Food and mealtime are significant aspects of home life and cultural identity. Evergreen Community Care's mealtime standards:

- Meals are served in a consistent, home-like setting; wherever possible, staff and youth eat together
- Youth are involved in age-appropriate meal planning, grocery shopping, and cooking as life skills development
- Dietary restrictions and preferences based on religion, culture, allergy, or medical need are documented and strictly accommodated
- Food is never withheld as a consequence; food is not used as a reward
- Youth who choose not to eat at mealtimes are offered food at the next meal without commentary; restrictive or disordered eating is flagged with the health care team
- Hot beverages served to youth do not exceed 60°C to prevent scalding; all hot liquids are assessed before being provided to youth — Residential Care Regulation amendment, April 2026

D.8 Personal Property and Privacy

Youth have the right to reasonable privacy and possession of their personal belongings:

- Youth's personal belongings are listed in the admission inventory and stored safely in their bedroom or a designated secure area
- Staff do not enter a youth's bedroom without knocking and receiving permission, except in an emergency or for scheduled wellness checks
- Youth's private correspondence (letters, diary, personal notes) is not read without the youth's knowledge or a specific safety justification documented in the shift log
- Lost, damaged, or stolen personal property is reported to the Program Manager and the youth's social worker; replacement is arranged where possible

STANDARD E — CHILD AND YOUTH SERVICE PLANNING

Regulatory Reference: MCFD Standards E.1–E.7; CFCSA s.70(1)(b); MCFD Child Welfare Policy Chapter 4

E.1 Individual Service Plan (ISP) — Overview

Every child and youth placed at Evergreen Community Care has an Individual Service Plan (ISP) that guides all aspects of their care. The ISP is developed collaboratively with the youth, their MCFD social worker, family (where involved), and all relevant professionals. It is a living document that is reviewed and updated regularly.

E.2 ISP Development and Components

ISP Component	Content and Requirement
Strengths and interests	Youth's identified strengths, interests, cultural background, and personal goals — developed with the youth's active input
Goals	Short-term and long-term goals across domains: safety, health, education, social/emotional, cultural, independent living
Care plan activities	Specific, measurable activities to support each goal; responsibilities of Evergreen Community Care, MCFD, family, and the youth
Permanency goal	The long-term permanency goal as established by MCFD (e.g., reunification, guardianship, adoption, agreement with young adult)
Education plan	Current school or educational program; attendance; academic support; IEP if applicable
Health care plan	Medical, dental, mental health, and medication management; appointment schedule
Cultural plan	For Indigenous youth: Cultural Plan developed within 30 days of placement; for all youth: cultural identity and heritage support
Safety plan	Identified risks; specific safety strategies; supervision level requirements; physical environment modifications
Behaviour support	Identified behaviour patterns, triggers, de-escalation strategies; restraint authorization if applicable
Contact plan	Family and significant others contact schedule; any court orders or restrictions on contact
Transition plan	Steps toward the permanency goal; skills development for transition

E.3 ISP Review Timelines

Trigger	Review Required
Initial ISP (new placement)	Developed within 14 days of placement; preliminary plan within 48 hours

Regular review	At minimum every 3 months; or at each MCFD Case Conference
Significant change in youth's status	Within 5 business days of: new diagnosis, medication change, major incident, significant behavioural change, school change
Placement breakdown risk	Immediately upon identification of placement stability concerns
Approaching transition	ISP includes formal Transition Plan 6 months before anticipated discharge or transition
Youth's birthday (16th and 17th)	ISP review specifically addresses Agreements with Young Adults, transition to independence planning, and life skills goals

E.4 Youth Participation in Service Planning

Meaningful youth participation in their own service planning is a legal requirement under the CFCSA and a core value of Evergreen Community Care:

- Youth are invited to attend their own ISP and care planning meetings; attendance is supported by staff who help the youth understand the process and prepare
- Youth who choose not to attend have their views gathered beforehand and presented at the meeting
- ISP goals and language are accessible and meaningful to the youth; youth receive a copy of their own ISP in an accessible format
- Youth feedback about their care and goals is documented in the shift log and incorporated into ISP updates
- Youth are informed of and involved in transitions and placement changes as early as possible; abrupt moves without youth preparation are avoided wherever possible

E.5 Care Team Coordination

The ISP is implemented through a coordinated care team approach:

- Care team members typically include: the youth (lead); MCFD social worker; Program Manager; direct care staff; family (where involved); school (where applicable); health care providers; cultural supports for Indigenous youth
- Evergreen Community Care's Program Manager is the primary point of contact for the care team and facilitates communication between team members
- Information about the youth is shared within the care team on a need-to-know basis consistent with FOIPPA
- Significant changes to the care plan or ISP require MCFD social worker approval before implementation, unless it is an emergency safety measure

E.6 Transition Planning

Transitions — whether between placements, back to family, or to independent living — are planned carefully and supported actively. The Transition Plan (Appendix G) includes: the reason and timeline for transition; the youth's goals and concerns; life skills assessment; connections established (community, school, health providers, cultural); and supports in place after the transition. Transition is never abrupt.

STANDARD F — SHELTER AND EQUIPMENT

Regulatory Reference: MCFD Standards F.1–F.4; Residential Care Regulation s.22–56; BC Building Code; BC Fire Code

F.1 Physical Facility Standards

Every Evergreen Community Care residential facility meets the following physical standards, confirmed annually through licensing inspection and monthly through internal safety inspection:

Physical Standard	Requirement
Bedrooms	Each youth has their own bedroom or a shared room with an assigned housemate of comparable age and compatibility; minimum floor area per youth per RCR; adequate natural light; safe and lockable from the inside only
Bathrooms	Appropriate number of bathrooms per youth population; maintained in clean, hygienic condition; privacy ensured; grab bars where required
Common areas	Comfortable, welcoming living and dining areas; adequate for the number of residents; age-appropriate furniture and decor
Kitchen	Fully equipped residential kitchen; food storage adequate for the number of residents; fire suppression over stove; all appliances maintained in good repair
Outdoor space	Safe, accessible outdoor space available to youth; outdoor furniture; equipment appropriate to age range
Accessibility	Physical facility accommodates any identified mobility or accessibility need of current residents
Temperature	Facility maintained at a comfortable indoor temperature throughout the year; minimum 20°C in occupied rooms in winter
Water temperature	Hot water at point of use does not exceed 49°C (120°F) per RCR amendment, April 2026; thermostatic mixing valves installed where required
Laundry	Laundry facilities available for youth's personal laundry; youth are supported to launder their own clothing as a life skill
Storage	Adequate storage space for each youth's personal belongings; a secure locked space for personal items of value

F.2 Equipment and Furnishings

All equipment and furnishings in the facility are:

- In good repair; damage is reported and repaired promptly
- Safe for the age range and behaviour profile of youth currently placed
- Not used as consequences; furniture, bedding, and personal comfort items are not removed as consequences

- Inclusive and culturally reflective where possible; decor and art represent diversity and the cultures of youth in care

F.3 Facility Inspection and Maintenance

Inspection Type	Frequency	Responsibility
Monthly safety inspection (MCFD/RCR standard)	Monthly	Program Coordinator / JHSC Member
Fire safety inspection	Monthly (plus annual by fire authority)	Program Manager
Annual licensing inspection	Annually	Health Authority Licensing Officer
Annual OHS compliance audit	Annually	Health and Safety Coordinator
Post-incident facility check	Within 24 hours of any significant incident	Program Manager
Pre-occupancy inspection (new placements)	Before admission of a new resident whose profile requires environmental modifications	Program Manager

F.4 Emergency and Safety Equipment

The following emergency and safety equipment is maintained at each facility in compliance with BC Fire Code, OHS Regulation, and MCFD Standards:

☐ Smoke detectors — all sleeping areas and common rooms	☐ CO detectors — all occupied levels
☐ Fire extinguishers — minimum one per floor	☐ First aid kit — Type 2 minimum
☐ Emergency lighting — all exits and stairways	☐ Emergency phone or communication device
☐ Evacuation plan — posted on each floor	☐ Earthquake emergency kit (72-hour supplies)
☐ Vehicle first aid kit	☐ Personal alarm device for working-alone shifts

STANDARD G — HUMAN RESOURCES

Regulatory Reference: MCFD Standards G.1–G.4; Residential Care Regulation s.66–80; Criminal Records Review Act; BC Human Rights Code; Employment Standards Act

G.1 Hiring and Screening Standards

All persons engaged to work with children and youth at Evergreen Community Care meet the following minimum requirements before beginning any duty:

Requirement	Detail
Criminal Record Check — Vulnerable Sector	All staff, volunteers, contractors, and students hold a current, satisfactory Vulnerable Sector CRC before beginning any work with youth; renewed every 5 years or upon a relevant conviction
Reference checks	Minimum two professional references checked for all new employees; at least one from a direct supervisor in a previous role with children or youth
Mandatory training completion	OHS orientation; WHMIS; Workplace Violence Prevention; Non-Violent Crisis Intervention (or equivalent) — completed before first solo shift
Qualifications	Youth Care Workers: diploma or degree in Child and Youth Care, Social Work, or related field; or equivalent experience with demonstrated competency; per facility job description
First aid certification	Minimum Emergency First Aid; minimum one Occupational First Aid Level 2 designate per shift
Driver's licence (where applicable)	Valid BC Class 5 driver's licence; satisfactory driver's abstract; no more than 2 at-fault incidents in 3 years
Health requirements	Health clearances as required by MCFD or licensing; tuberculosis screening where indicated

G.2 Staffing Ratios and Deployment

Staffing ratios are governed by the Residential Care Regulation, MCFD Standards, and organizational policy. Full staffing ratio requirements and deployment guidelines are described in Section 26 of this Manual. At minimum:

- A minimum of one awake staff is present and responsible for the safety of youth at all times during waking hours
- An awake, on-site staff member is present throughout all overnight hours
- Ratios are adjusted above regulatory minimums where individual youth risk levels or profiles require enhanced staffing
- Staff may not be left alone to care for more youth than is safe given the current cohort's needs, regardless of whether the regulatory ratio is technically met

G.3 Staff Training Requirements

All direct care staff complete the following mandatory training, with timelines and renewal requirements tracked in the Staff Training Compliance Matrix (Appendix L):

Training	Who	Timeline	Renewal
OHS Program orientation	All staff	Before first shift	Review at annual update
Residential Care Program Manual orientation	All staff	Before first solo shift	At each major update
Non-Violent Crisis Intervention (NVCI) or equivalent	All direct care staff	Within 90 days; before solo shifts	Annual
Indigenous Cultural Safety and Humility	All staff	Within 60 days of hire	Annual
Gender-Affirming Care training	All staff	Within 60 days of hire	Annual
Trauma-Informed Care foundations	All direct care staff	Within 60 days	Every 2 years
Medication Administration	Authorized staff	Before any medication duty	Annual competency check
First Aid CPR Level C	All direct care staff	Within 30 days	Annual
Mandatory Reporting (CFCSA s.14)	All staff	Before first shift	Every 2 years
Suicide and Self-Harm Response	All direct care staff	Within 60 days	Annual
Working Alone safety	Staff on solo overnight shifts	Before first solo shift	Annual
WHMIS 2015	All staff	Before first shift	Every 3 years

G.4 Performance Management and Supervision

Performance management at Evergreen Community Care is described in full in the HR Policy Binder (ECC-HR-POL-2026-001). For direct care staff, the following supervision structures apply:

- Regular one-on-one supervision: at minimum monthly for all direct care staff; more frequently for staff in their probationary period or who have identified performance or wellbeing concerns
- Reflective supervision: a component of supervision focused on the staff member's emotional responses to the work, secondary trauma, and practice reflection — see Section 28
- Annual performance review: completed each year by the Program Manager; includes self-assessment, goal-setting, and training plan
- Progressive discipline: applied consistently and in accordance with the HR Policy Binder for performance and conduct concerns; due process is always observed

STANDARD H — ORGANIZATION AND MANAGEMENT

Regulatory Reference: MCFD Standards H.1–H.2; CCALA; Residential Care Regulation s.7–21

H.1 Governance and Accountability Structure

Governance Element	Detail
Board of Directors	Provides governance and oversight; approves major policies; ensures organizational accountability; receives annual program review
Executive Director	Accountable for all operations, policy, and MCFD contract compliance; primary signing authority; approves all program policies
Program Manager	Responsible for day-to-day operation of residential programs; primary MCFD contact; supervises direct care staff
Senior Youth Care Worker / Coordinator	Shift lead; supervises team during shift; first point of escalation for operational concerns
Youth Care Worker	Provides direct care to youth; implements the ISP and care plan
Organizational chart	Current chart maintained and provided to MCFD upon request or following any organizational change

H.2 Administrative Requirements

- This Manual constitutes the written policy and procedure manual required by MCFD Standards H.1; it is reviewed annually and upon any significant change to MCFD Standards or applicable legislation
- All MCFD reporting requirements are met on time; contact information for all managerial staff is current in the MCFD contractor database
- Financial records are maintained in accordance with GAAP; annual financial reports are provided to MCFD as required
- Insurance coverage is maintained at required levels; policies are reviewed annually; MCFD is named as an additional insured where required
- All vehicles used to transport youth are insured, inspected, and compliant with the Motor Vehicle Act
- The organization maintains current registration in good standing with the BC Registry of Societies

H.3 Annual Program Review — Standard H.2

Evergreen Community Care conducts a formal annual program review each July in accordance with MCFD Standard H.2. The review is guided by the QA Program Binder (ECC-QA-2026-001) and addresses:

- Relevance and effectiveness of the program in meeting youth's needs
- Achievement of goals from Individual Service Plans across the program population
- Key performance indicators: safety incidents, training compliance, licensing compliance, occupancy, staff turnover

- Youth and family feedback from annual surveys
- Staff satisfaction data
- Recommendations for program improvements in the coming year

The annual review report is submitted to MCFD, shared with staff at an all-staff meeting, and presented to the Board of Directors. Significant findings and recommended changes are communicated to all affected parties within 30 days of the review.

H.4 Code of Ethics (Required by H.1)

Evergreen Community Care's Code of Ethics is described in Section 5 of this Manual and addresses all areas required by MCFD Standard H.1, including: conflicts of interest; workplace behaviour and relationships; gifts; religious and cultural influence; remuneration; and confidentiality. The Code is reviewed annually and incorporated into staff orientation.

PART III — OPERATIONAL PRACTICE STANDARDS

6. ADMISSION, INTAKE, AND ORIENTATION

Regulatory Reference: RCR s.7–10; MCFD Standards A.1, E.1; CFCSA s.70

6.1 Pre-Admission Process

Before accepting a new placement, the Program Manager reviews available information about the referred youth to confirm:

- The placement is made by or with MCFD or DAA authorization
- The facility has licensed capacity
- The youth's assessed needs are within the program's current capacity to meet safely
- The placement does not compromise the safety of current residents
- Any environmental modifications required for this youth's safety can be made before arrival

For emergency placements where full information is not available, the Program Manager accepts the placement based on available information and initiates a safety-focused ISP review within 48 hours.

6.2 Admission Checklist

The following tasks are completed at or before admission (Appendix A — Admission and Intake Checklist):

Task	Timeline	Responsible
Youth arrives and is introduced to staff and other residents	Day 1	Admitting staff
Bedroom assigned and youth's belongings inventoried	Day 1	Admitting staff
Rights information reviewed with youth orally; handout provided (Appendix C)	Day 1 within 4 hours	Admitting staff
Medical information and emergency contacts confirmed	Day 1	Admitting staff
Current medications inventoried; MAR initiated	Day 1	Medication-authorized staff
Preliminary risk assessment and safety plan reviewed	Day 1	Program Manager
Emergency contact information for social worker confirmed	Day 1	Admitting staff

Relevant court orders or access restrictions confirmed	Day 1	Program Manager
Hot water temperature at point of use verified (max 49°C)	Day 1	Program Manager
Youth oriented to the home: rules, routines, emergency procedures	Within 48 hours	Designated staff + New Youth Orientation Guide, Appendix B
Preliminary Individual Service Plan developed	Within 48 hours	Program Manager
Cultural background documented; Indigenous status confirmed; Cultural Plan initiated	Within 48 hours	Program Manager
Full ISP developed collaboratively	Within 14 days	Program Manager + MCFD Social Worker

6.3 Youth Orientation Program

Every new youth receives a structured orientation to the home within the first 48 hours, using the New Youth Orientation Guide (Appendix B). Orientation is age-appropriate, accessible, and delivered at the youth's pace:

- Introduction to all current residents and staff on shift; staff photo board available
- Tour of the facility including: their bedroom, bathroom, common areas, kitchen, outdoor space, exits, and emergency equipment locations
- Explanation of daily routine: mealtimes, wake-up, bedtime, school/activities, recreational time
- House expectations: community standards for respectful co-living, not a rulebook of punishments
- Rights overview: verbal review of rights (Appendix C); youth informed of how to access the Advocate
- Emergency procedures: fire exits, assembly point, who to call in an emergency
- Contact information: staff contacts; their social worker's contact; the Advocate
- Questions: explicit invitation to ask anything they want to know

Orientation is documented in the admission record and signed by the youth (or documented if the youth declines to sign).

7. INDIVIDUAL SERVICE RECORDS AND DOCUMENTATION

Regulatory Reference: RCR s.10–16; MCFD Standards E.7, H.1; FOIPPA

7.1 Required Records for Each Youth

Record Type	Content and Retention
Admission record	Placement date, MCFD social worker, reason for placement, legal status, emergency contacts, demographic information
Individual Service Plan (ISP)	Current and all previous versions; retained for 7 years after discharge
Shift log / daily notes	Factual, objective daily notes covering all significant events, interactions, activities, incidents; retained 7 years
Medication Administration Record (MAR)	Complete MAR for all medications; administered/refused entries; retained 7 years
Health care log	Medical appointments, diagnoses, treatments, communications with health care providers; retained 7 years
Education log	School attendance, communications with school, IEP-related notes; retained 7 years
Incident reports	All Critical Incident Reports and internal incident reports; retained permanently
Behaviour Support Plan	Current BSP and all prior versions; retained 7 years
Contact log	Family contact — date, duration, who, nature, youth's response; retained 7 years
Allowance and financial records	Youth's allowance payments, trust funds, personal purchases; retained 7 years
Court orders and legal documents	Copies of all orders affecting the placement; original held by MCFD
Cultural Plan	For Indigenous youth; current and all prior versions; retained 7 years
Transition plan and discharge summary	Completed at discharge; includes youth's destination, supports in place; retained 7 years

7.2 Documentation Standards

All documentation at Evergreen Community Care meets the following standards:

- **Accurate:** documentation reflects what actually happened, observed, or was said — not assumptions, interpretations, or guesses about motivation
- **Objective:** language is factual and non-judgmental; describes behaviour and events in specific, observable terms
- **Contemporaneous:** entries are made as close in time to the event as possible; shift logs are completed before staff leave the facility

- Complete: significant events are not omitted from documentation, even if the event reflects poorly on the program or a staff member
- Legible: handwritten records are legible; electronic records are spell-checked
- Signed and dated: all entries include the staff member's name (printed and signed) and the date and time

Documentation that is falsified, altered, or omitted is a serious disciplinary and potentially legal matter. Staff are encouraged to document accurately even when an event is embarrassing or reflects a mistake.

7.3 Shift Log — Required Daily Entries

The shift log is completed at the end of every shift for every youth in the home. Minimum daily entries include:

- Youth's general mood and presentation throughout the shift
- Meals eaten (what, how much, any refusals or concerns)
- School attendance or educational activity
- Medications administered per MAR (reference MAR; do not duplicate in log unless there was an exception)
- Any appointments attended
- Recreation and activities
- Family or significant person contact
- Any behaviour of note — positive or challenging — with factual description
- Any incident — even if handled without issue — with reference to incident report if completed
- Any concerns brought forward to the next shift
- Name and signature of staff completing the entry

7.4 Access to and Confidentiality of Records

Individual service records are confidential. Access is restricted to:

- Evergreen Community Care staff with a current need to know for care purposes
- MCFD staff in their role managing the child's care
- Health Authority Licensing Officers in their licensing function
- The youth themselves (age-appropriate access; supported by staff where helpful)
- Legal guardians of youth who are not in permanent government care

Requests for records from others — including family members, police, courts, or other organizations — are referred to the Program Manager, who consults the Executive Director and, if necessary, legal counsel before any disclosure.

8. CARE PLANNING AND INDIVIDUAL SERVICE PLAN (ISP) PROCESS

Regulatory Reference: MCFD Standards E.1–E.7; CFCSA s.70(1)(b); MCFD Child Welfare Policy Chapter 4

The ISP process is described in full under Standard E (Section E.1–E.6). This section provides the operational procedure for how Evergreen Community Care fulfils its ISP obligations.

8.1 ISP Development Process

Phase	Process
Preliminary plan (within 48 hours)	Program Manager reviews available information; identifies immediate safety needs, health needs, and essential contacts; documents in a preliminary care plan shared with social worker
Strengths and needs assessment	Direct care staff contribute observations during the first 2 weeks; youth's MCFD assessment is reviewed; youth is engaged in identifying their own goals and preferences
ISP meeting	Convened within 14 days of placement; includes youth, MCFD social worker, Program Manager, family (where appropriate and safe), and relevant others; youth's goals are placed at the centre
ISP document	Draft prepared by Program Manager; reviewed with youth in accessible language; approved by MCFD social worker; signed by youth (where able and willing), Program Manager, and MCFD SW
Implementation	Distributed to all direct care staff; morning meeting / shift handover includes ISP goals and current priorities; care is guided by the ISP
Monitoring	Progress toward ISP goals noted in shift logs; flagged concerns communicated to Program Manager
Review	Formal review at minimum every 3 months or at Case Conference; also triggered by significant events

8.2 ISP Goal Domains

ISP goals are developed across the following domains, ensuring holistic care:

Domain	Example Goals
Safety	Reduce elopement incidents; develop safety skills; implement environmental safety modifications
Health	Attend all medical appointments; establish with a physician; support healthy eating and exercise
Emotional wellbeing	Engage in counselling; develop 2 coping strategies for identified triggers; reduce self-harm incidents
Education	Attend school 4+ days per week; complete homework with support; develop a career interest

Social and relational	Develop one positive peer relationship; participate in a community group or activity
Cultural	Attend monthly cultural event; connect with an Elder; develop knowledge of their cultural heritage
Family	Maintain weekly contact with family member; work toward safe family visit
Independent living	Develop cooking skills; manage own laundry; open a bank account; develop a budget
Transition	Identify 3 post-placement supports; connect with Foundry or other community resources

9. HEALTH CARE MANAGEMENT

Regulatory Reference: MCFD Standards D.1.2, D.1.3, E.2; RCR s.36–45; Residential Care Regulation

9.1 Health Care Overview

Evergreen Community Care is responsible for ensuring that every youth in its care has access to timely, appropriate health care. The organization acts in the role of the responsible adult for health care decisions within the limits of its MCFD service agreement and applicable legislation.

9.2 Medical Needs Assessment and Health Care File

- A health care file is established for each youth at admission, containing: known medical history; current diagnoses; allergies; current medications; immunization record; name of family doctor, dentist, and other regular health care providers
- The health care file is updated at every appointment and whenever there is a new diagnosis, medication change, or health concern
- The MCFD social worker is informed of any new health diagnosis, significant medication change, or health event requiring emergency care within 24 hours
- Each youth's health care needs are summarized in their ISP and updated at each ISP review

9.3 Access to Health Care

Care Type	Standard
Family physician	Every youth is established with a family physician or nurse practitioner; where no physician is immediately available, a walk-in clinic or Nurse Practitioner clinic is used; the social worker is asked to assist with physician connection
Dental care	Annual dental checkup arranged for all youth; dental pain or emergency is treated promptly regardless of scheduled appointment status
Mental health care	Youth with identified mental health needs are connected with appropriate services (psychiatry, psychology, counselling); appointments are attended and followed up
Specialist care	Referrals arranged as required; appointments attended; reports filed in health care file; MCFD and prescribers informed
Emergency care	911 is called immediately for any medical emergency; staff accompany youth to emergency care; the social worker is notified within 1 hour
Vision care	Vision needs are identified and glasses or other corrective aids are obtained; youth with unidentified vision issues referred for assessment
Immunizations	Immunization records are reviewed at admission; any gaps are addressed in consultation with the family physician and the social worker
Reproductive health	Youth are supported non-judgmentally in accessing sexual and reproductive health care; parental/guardian consent requirements followed per applicable law

9.4 Mental Health Support

Many youth in residential care have diagnosed or undiagnosed mental health conditions resulting from trauma, adverse childhood experiences, and disrupted attachment. Evergreen Community Care provides a therapeutic care environment as a foundation for mental health, supplemented by:

- Access to counselling — connection to a therapist or counsellor is prioritized at admission for youth without existing mental health support
- Psychiatric consultation — arranged through the MCFD social worker or via the youth's family physician for youth with complex mental health needs
- Trauma-informed daily care — the program's daily interactions and routines are designed around trauma-informed principles (see Section 25)
- Crisis response — staff are trained to respond to mental health crises including self-harm and suicidal ideation (see Section 17)
- Medication management — psychiatric medications are administered as prescribed and monitored for efficacy and side effects (see Section 10)

9.5 Consent to Health Care

Authority to consent to health care for youth in MCFD care is governed by the CFCSA:

- Where MCFD holds custody, the Director (or delegated MCFD social worker) consents to health care decisions beyond routine care
- Routine and necessary care does not require advance MCFD consent; significant, elective, or irreversible health care decisions do
- Youth 16 and older may consent to their own health care in most circumstances; staff support their right to consent or decline with full information
- Evergreen Community Care consults the MCFD social worker before consenting to any non-emergency elective procedure, prescription of a new psychiatric medication, or other significant health care decision

10. MEDICATION ADMINISTRATION PROGRAM

Regulatory Reference: RCR s.46–52; MCFD Standards D.1.3; Nurses and Nurse Practitioners Act; Drug Schedules Regulation

10.1 Authorization to Administer Medications

Medications are administered only by staff who:

- Hold current Medication Administration authorization on file, confirmed by competency assessment
- Are on duty and directly responsible for the youth at the time of administration
- Have been briefed on any changes to the youth's medication at the shift handover

Staff who have not completed Medication Administration training must not administer any medication to any youth under any circumstances, including in an emergency. In an emergency requiring immediate medication (e.g., EpiPen administration), the trained staff member administers the medication or 911 is called immediately.

10.2 Medication Storage

Medication Type	Storage Requirement
All medications	Locked cabinet accessible only to authorized staff; key controlled and accounted for at every shift change
Schedule I and II controlled substances (opioids, stimulants, benzodiazepines)	Double-locked storage (e.g., locked box within locked cabinet); counted and recorded at every shift change; inventory discrepancies reported to Program Manager and social worker immediately
Temperature-sensitive medications	Refrigerated at 2–8°C in a locked refrigerator or locked section of the facility refrigerator; temperature logged daily
Emergency medications (EpiPen, Diastat)	Stored in a clearly labeled, immediately accessible location known to all staff; location reviewed at every shift change; one designated staff at all times knows exact location
Expired medications	Removed from the active medication cabinet immediately; returned through the BC Medications Return program; never disposed of in general waste

10.3 Medication Administration Procedure

Step	Required Action
1. Confirm identity	Confirm the youth's identity before every administration; do not rely on memory alone
2. Verify order	Check the Medication Administration Record (Appendix F) against the current prescription; confirm medication name, dose, route, time, and any special instructions
3. Check for interactions	Be aware of any allergies documented in the health file; follow any

	prescriber notes about administration conditions
4. Prepare	Prepare the medication in a clean area; do not prepare more than one youth's medications at a time
5. Administer	Administer directly to the youth; confirm ingestion for oral medications; do not leave medications unattended with the youth
6. Document	Record on the MAR immediately after administration: date, time, medication, dose, route, and initials; do not pre-sign the MAR
7. Refused medication	If the youth refuses, document the refusal on the MAR; notify the Program Manager; notify the prescriber if the medication is scheduled for a condition that may deteriorate without the dose
8. Suspected non-ingestion	If there is reason to believe a youth did not swallow an oral medication, notify the Program Manager; prescriber consulted if clinically significant
9. Error	Any medication error (wrong drug, wrong dose, wrong time, wrong person) is treated as an immediate medical concern; 911 called if indicated; prescriber called; incident report completed; social worker notified

10.4 Controlled Substance Count Procedure

At every shift change, outgoing and incoming staff jointly count all controlled substances:

- Count is conducted together, by both staff members simultaneously
- Count is reconciled against the last recorded count on the MAR
- Any discrepancy — even by one dose — is reported to the Program Manager immediately; a full investigation is initiated
- The count and any discrepancy note are signed by both staff in the count log

11. BEHAVIOUR SUPPORT AND POSITIVE PROGRAMMING

Regulatory Reference: MCFD Standards C.6, D.1.4, D.1.7; RCR s.74–78; CFCSA s.70

11.1 Philosophy

Behaviour support at Evergreen Community Care is grounded in the understanding that all behaviour communicates an underlying need. Our role is to understand the need beneath the behaviour and help the youth develop healthier ways to meet that need. Punishment-based approaches are inconsistent with trauma-informed care and with the MCFD Standards.

11.2 Behaviour Support Plan (BSP)

Every youth who presents with behaviours that challenge the safety or functioning of the household, or who has a history of significant behavioural incidents, has a written Behaviour Support Plan (Appendix E). The BSP:

- Is developed with the youth's meaningful input; uses the youth's own language where possible
- Identifies the youth's specific behaviour patterns, including antecedents and triggers
- Identifies the function of the behaviour (what need it is meeting)
- Documents specific, agreed de-escalation strategies that are known to work for this youth
- Identifies replacement behaviours and the skills development needed to build them
- Is reviewed with all staff at each shift handover when there is a high-risk period
- Is reviewed and updated at each ISP meeting and following any significant behavioural incident

11.3 De-Escalation Framework

Stage	Signs	Staff Response
Baseline (calm)	Youth is regulated, engaged, and within normal range for them	Relationship-building; positive reinforcement; skill development; proactive BSP review
Agitation (triggered)	Increased motor activity; short attention span; repetitive questions; voice change; withdrawal	Acknowledge feeling; reduce demands; offer choice; provide space; activate BSP strategies
Acceleration (escalating)	Focused anger; challenging or threatening statements; property disruption	Calm, low-stimulation environment; directive language only as needed; call for backup; do not engage in power struggle
Peak (crisis)	Active aggression; self-harm; extreme emotional dysregulation	Safety first; backup on scene; minimal verbal engagement; physical intervention if life-safety threshold met (see Section 19)
De-escalation	Reducing aggression; beginning to respond to staff	Maintain calm; do not debrief immediately; allow time and

		space to regulate
Recovery (calm restored)	Youth is regulated and accessible	Welfare check; brief verbal acknowledgement; full debrief when ready; document; review BSP

11.4 Prohibited Behaviour Management Practices

⚠ The following practices are PROHIBITED under MCFD Standards, the CFCSA, and Evergreen Community Care policy. Use of any prohibited practice is a serious disciplinary matter and a critical incident requiring immediate reporting to MCFD.

- Corporal punishment (any physical contact as a consequence, including spanking, slapping, pinching)
- Seclusion (placing a youth in a room and preventing them from leaving) — distinct from the youth voluntarily choosing to go to their room
- Restraint used as a consequence, punishment, or for staff convenience rather than imminent life safety
- Withholding meals, bedding, or basic care needs as a consequence
- Removing a youth's right to family contact as a consequence
- Humiliating, degrading, or shaming a youth
- Verbal threats or intimidation
- Any technique that restricts breathing or places pressure on the neck, chest, or abdomen
- Use of any restraint technique not covered in the organization's approved physical intervention training

11.5 Positive Programming and Skill Development

Positive programming is the daily work of residential care: creating opportunities for youth to develop skills, experience success, and build a positive self-identity. Examples include:

- Cooking and household management: youth cook meals, do laundry, care for the household — not as chores for compliance but as genuine life skills
- Community access: regular community outings that build real-world skills and social confidence
- Hobbies and recreation: identification and active support for each youth's personal interests
- Volunteering and contribution: opportunities for youth to contribute to their community and develop a sense of agency
- Celebrations: birthdays, achievements, milestones, and cultural celebrations are observed meaningfully

12. EDUCATION SUPPORT AND SCHOOL LIAISON

Regulatory Reference: MCFD Standards D.1.6, E.2; CFCSA s.70(g); BC School Act

12.1 School Enrollment

- Every school-aged youth is enrolled in their catchment school or an approved educational alternative within 5 business days of placement
- If enrollment is delayed for any reason (records, IEP planning), the Program Manager notifies the MCFD social worker within 48 hours and provides interim educational support in the home
- School transfers are managed collaboratively with the school district; previous IEP records are requested and transferred immediately upon placement
- Where a youth's behaviour history requires a modified school start or transition plan, this is developed in collaboration with the school principal, MCFD social worker, and support team

12.2 Attendance Support

- Staff prepare youth for school daily: appropriate clothing, breakfast, school materials prepared the night before
- Absences are tracked in the education log; more than 2 unexcused absences in any 2-week period triggers a conversation with the school and the MCFD social worker
- Staff address barriers to attendance — anxiety, peer conflict, learning difficulties, unsafe clothing, missing supplies — with the youth and the school proactively
- School refusal is treated as a sign of distress, not defiance; the underlying cause is investigated and addressed through the BSP and ISP

12.3 School Communication and Advocacy

- Staff attend school meetings (parent-teacher conferences, ISP/IEP meetings, case conferences) when invited and appropriate
- Significant school events (suspensions, academic concerns, positive achievements) are documented in the shift log and communicated to the MCFD social worker within 24 hours
- Staff advocate with schools for each youth's educational needs; MCFD is asked to support advocacy where the school is not responsive
- Youth who have been identified as needing IEP support are advocated for to receive it promptly

12.4 Homework and Academic Support

- A consistent daily homework time is built into the home routine
- A quiet, adequately lit workspace with necessary materials (pens, paper, calculator) is provided
- Staff provide homework support within their ability; tutoring is arranged for youth who need specialized academic support
- Youth's academic achievements are celebrated; grades and report cards are reviewed positively

13. FAMILY CONTACT AND RELATIONSHIP MAINTENANCE

Regulatory Reference: MCFD Standards D.1.9, E.1; CFCSA s.70; Child in Care Regulation s.5

13.1 Philosophy

Family connection is a fundamental aspect of a child's identity and wellbeing. Evergreen Community Care supports meaningful family contact as a matter of right, consistent with each youth's care plan and any applicable court orders. Staff do not allow their personal views about a family to influence the quality of support they provide for family contact.

13.2 Family Contact Plan

- Each youth's ISP includes a Contact Plan specifying: who the youth may have contact with; the frequency and type of contact; any supervision requirements; and any court-ordered restrictions
- The Contact Plan is developed in consultation with the MCFD social worker; changes require social worker approval
- In the absence of any restriction, the default is to support the youth's contact with family members they want to contact
- Court orders restricting contact are followed strictly; any question about a court order's interpretation is referred to the MCFD social worker before the contact occurs or is refused

13.3 Facilitating Family Contact

- Phone and video contact with approved family members is facilitated during reasonable hours without staff monitoring (unless the Contact Plan specifies supervision)
- In-person visits occur in a comfortable, welcoming environment; the home is presented as a place of genuine care, not a custodial institution
- Where visits require supervision per the Contact Plan, supervision is provided by a staff member who can maintain a warm, non-adversarial presence with the family
- Family members who visit are treated with dignity and respect; staff do not discuss other youth's information with visiting family members
- Transportation to family visits is arranged where required and within organizational capacity; MCFD is asked to arrange transportation when it is beyond the program's capacity

13.4 Documenting Contact

All family contact is documented in the Contact Log:

- Date, time, type of contact (phone, video, in-person)
- Who the youth had contact with
- Duration of contact
- Youth's mood and response before, during, and after contact (objective description)
- Any concerns arising from the contact (e.g., youth was distressed following the call; family member made inappropriate statements)

Significant concerns about the impact of family contact on a youth's wellbeing are communicated to the MCFD social worker within 24 hours with documentation.

14. COMMUNITY ACCESS AND LIFE SKILLS DEVELOPMENT

Regulatory Reference: MCFD Standards D.1.5, D.1.7; CFCSA s.70(g); RCR s.7

14.1 Community Access as a Right

Youth have the right to participate in social and recreational activities in the community. Evergreen Community Care actively facilitates community access as a developmental priority, not a privilege contingent on behaviour.

14.2 Community Outing Planning

All community outings involving youth are planned using the Transport and Community Outing Safety Plan (Appendix K), which includes:

- Destination and activity description
- Risk assessment for the specific outing and group
- Staff accompanying and ratio maintained
- Youth participants identified
- Emergency contact information for all participants
- Communication device confirmed functional
- Special considerations from individual youth's care plans

14.3 Life Skills Development

Evergreen Community Care integrates life skills development into daily care. Life skills goals are documented in the ISP. The following life skills are systematically taught and practiced based on each youth's age, ability, and developmental readiness:

Life Skill Domain	Examples	ISP Integration
Home management	Cooking, cleaning, laundry, grocery shopping, budgeting	Documented in ISP; regular practice in the home
Financial literacy	Banking, budgeting, understanding bills, avoiding debt	ISP goal; supported by youth's allowance management
Health and self-care	Hygiene, sleep habits, healthy eating, accessing health care	ISP goal; modeled and supported by staff
Education and employment	Study skills, resume, job search, interview preparation	ISP goal; supported by school and employment programs
Transportation	Using public transit, navigating the city	Community outing; ISP goal for older youth
Social skills	Resolving conflict, maintaining friendships, dating safely	Role modelling; group discussions; counselling
Digital literacy	Safe internet use, email, online banking	ISP goal; supervised practice
Legal knowledge	Rights as a young person, interactions with police, tenancy rights	Age-appropriate discussion; Foundry referral for 16+

15. TRANSITION AND DISCHARGE PLANNING

Regulatory Reference: MCFD Standards E.5, E.6; CFCSA s.70; MCFD Child Welfare Policy — Transition to Adulthood

15.1 Purpose

Transitions — to another placement, back to family, or to independence — are among the highest-risk periods in a young person's care journey. Poorly managed transitions increase the risk of crisis, homelessness, and re-engagement with child welfare. Evergreen Community Care approaches all transitions with deliberate planning and sustained support.

15.2 Types of Transitions

Transition Type	Evergreen Community Care's Obligations
Placement transfer (to another foster or group home)	Minimum 2 weeks' notice to youth where possible; transition meeting with receiving caregiver; complete file transfer; youth's belongings transferred safely; positive framing and staff support
Return to family (reunification)	Supported through graduated contact; safety planning with MCFD; family's readiness confirmed; post-return support plan in place
Aging out (18th birthday without permanency plan)	Transition plan initiated at 16th birthday; Agreement with Young Adult (AWA) explored; life skills assessed and gaps addressed; housing, income, health, and community supports arranged before the youth's 18th birthday
Agreement with Young Adult (AWA)	Evergreen Community Care supports the youth to understand and choose an AWA if eligible; supports the youth through the AWA application process
Adoption or legal guardianship	Supports the transition with the youth's active involvement; coordinates information transfer with receiving caregiver or adoptive family
Youth-initiated departure (elopement, self-discharge)	Missing youth protocol activated (Section 18); welfare-focused approach on return; re-engagement with ISP and care plan

15.3 Transition Planning Process

Transition planning is initiated at least 6 months before an anticipated transition wherever possible:

- Transition Plan (Appendix G) initiated and included in the ISP
- Youth's goals, concerns, and priorities for the transition are identified through the ISP process
- Life skills assessment completed; specific skills development integrated into ISP goals
- Community connections established in advance: school, health care, cultural supports, peer networks
- Housing and income support arranged for youth transitioning to independence
- Referrals made to Foundry, employment programs, mental health services, and other community resources
- Post-transition follow-up planned: Evergreen Community Care maintains contact for at least 30 days post-transition where the youth consents

15.4 Discharge Summary

A Discharge Summary is completed for every planned discharge within 5 business days of the youth leaving:

- Dates of placement and discharge
- Summary of goals achieved during the placement
- Outstanding care needs and recommendations
- Connections and supports in place at discharge
- Youth's expressed wishes for their future

The Discharge Summary is provided to the MCFD social worker and to the youth (in an accessible format) within the same timeline.

PART IV — SAFETY, INCIDENTS, AND CRITICAL CARE

16. CRITICAL INCIDENT REPORTING AND RESPONSE

Regulatory Reference: MCFD Standards C.2, H.1; MCFD Critical Incident Reporting Policy (2024); Contracted Service Agreement

16.1 Purpose and Reporting Obligation

Evergreen Community Care is required to report Critical Incidents to the youth's MCFD social worker promptly. Accurate, timely reporting is a legal obligation and a quality assurance function — it allows MCFD to coordinate care and allows Evergreen Community Care to learn and improve.

⚠ Failure to report a Critical Incident to MCFD is a serious breach of the Contracted Service Agreement and the CFCSA. All staff are personally responsible for ensuring incidents they witness are reported through the organizational chain — but reporting to a supervisor does not relieve the obligation to ensure MCFD is notified.

16.2 Critical Incident Types and Reporting Timelines

Incident Category	Event Types	Reporting Timeline
Category 1 — Immediate (same day)	Death or life-threatening injury; hospitalization; allegation of abuse or neglect; suicidal crisis or attempt; self-harm resulting in injury; missing youth; police contact; use of physical restraint; traumatic exposure (fire, violence, disaster); school suspension	Verbal notification to MCFD social worker within 2 hours; written Critical Incident Report (Appendix D) within 24 hours
Category 2 — Urgent (next business day)	Marked behaviour change; medication error with medical attention; property damage over \$500; youth involvement in criminal activity; significant family crisis affecting the youth; unauthorized overnight	Written Critical Incident Report (Appendix D) by end of next business day
Category 3 — Routine (weekly report or next ISP review)	Minor behavioural incidents without safety implications; pattern concerns requiring monitoring; school attendance concern	Documented in weekly program report to MCFD social worker or at next ISP review

16.3 When to Contact MCFD After Hours

If a Category 1 incident occurs and the youth's social worker is unavailable, contact MCFD Provincial Centralized Screening (PCS):

- PCS 24-hour line: 1-800-663-9122
- Explain you are a residential care service provider reporting a critical incident involving a child in care
- Provide: your name; organization; the youth's name (not their care status); the nature of the incident; actions taken; current status
- Follow up with the youth's social worker on the next business day with the written Critical Incident Report

16.4 Critical Incident Report Requirements

Every Critical Incident Report (Appendix D) is:

- Factual and objective — describes what was observed, not conclusions about causes or motivations
- Written in first-person, past tense — 'I observed...' not 'The youth was being....'
- Completed by the staff member present during the incident or, where that staff member has left, by the most direct available witness
- Reviewed and co-signed by the Program Manager before submission to MCFD
- Filed in the youth's ISR; copy retained by the Program Manager

16.5 Critical Incident Review Process

Within 5 business days of every Category 1 incident:

- Program Manager reviews the incident report and all available information
- Contributing factors are identified: environmental, staffing, individual youth factors, ISP adequacy
- Changes required to prevent recurrence are documented with named owners and target dates
- JHSC is notified where the incident has occupational health and safety implications for staff
- ISP and BSP are reviewed for any required updates
- Review findings and corrective actions are tracked in the monthly Program Quality Review (Appendix J)

17. SELF-HARM AND SUICIDAL CRISIS PROTOCOL

Regulatory Reference: MCFD Standards C.1, C.2; MCFD Policy 5.9; BC Ministry of Health Suicide Prevention Framework

17.1 Training Requirement

All direct care staff complete a recognized suicide and self-harm response training (such as Applied Suicide Intervention Skills Training — ASIST — or equivalent) within 60 days of hire and renew annually. No staff member works independently with youth who have documented self-harm or suicidal ideation history without this training.

17.2 When to Activate This Protocol

This protocol is activated when a youth:

- Discloses suicidal thoughts, plans, or intent
- Is found having engaged in self-harming behaviour
- Makes statements or shows behaviour that staff reasonably interpret as indicating suicidal ideation or intent
- Has access to means and has recently experienced a significant loss or destabilizing event

Staff do not delay activating this protocol while seeking certainty. Concern is sufficient.

17.3 Immediate Response — Step by Step

Step	Required Action
1. Stay with the youth	Remain with the youth at all times; do not leave them alone; summon backup while staying present
2. Remove means	Remove access to identified means if possible without escalating the situation; do not engage in a physical struggle to remove means from a youth
3. Engage calmly	Speak in a quiet, calm, non-alarming manner; express genuine care without panic; 'I'm really concerned about you right now and I want to make sure you're safe'
4. Ask directly	Ask clearly and without euphemism: 'Are you thinking about ending your life?' — direct questions do not increase risk; they open conversation
5. Listen	Allow the youth to speak; do not reassure prematurely or minimize; do not problem-solve or argue
6. Call supervisor	Notify the on-call supervisor immediately; if any immediate danger to life, call 911 first then supervisor
7. Call 911 if needed	Call 911 if: the youth has harmed themselves; has taken an overdose; has a weapon or access to means they are actively using; or is in immediate danger
8. Notify MCFD	Notify the MCFD social worker (or PCS after hours: 1-800-663-9122) within 2 hours of any suicidal crisis, regardless of how it resolved

17.4 Post-Crisis Actions

- Complete Critical Incident Report within 24 hours
- Update Safety Plan in the ISP within 24 hours; review with MCFD social worker
- Environmental risk assessment: remove or secure any identified means per the updated safety plan
- Arrange or confirm mental health follow-up for the youth within 48 hours
- Offer EFAP and debrief to all staff involved before the end of the shift
- ISP and BSP reviewed and updated within 5 business days

17.5 Ongoing Safety Planning

Every youth with a documented history of self-harm or suicidal ideation has a written Safety Plan as part of their ISP, which includes:

- Warning signs specific to this youth
- Coping strategies the youth has identified as helpful
- People the youth can talk to (internal and external)
- Environmental safety modifications in place at the facility
- Triggers and high-risk periods known for this youth
- The youth's own reasons for living (developed with the youth)
- Crisis resources posted for the youth: Crisis Centre BC (1-800-784-2433); Kids Help Phone (1-800-668-6868 / text HOME to 686868)

18. MISSING YOUTH PROTOCOL

Regulatory Reference: MCFD Missing Child/Youth Protocol (revised May 2025); MCFD Standards C.2; PCS: 1-800-663-9122

18.1 Definition

A youth is 'missing' when their whereabouts are unknown and they cannot be located after taking the same actions a prudent and responsible parent would take. There is no mandatory waiting period.

18.2 Phase 1 — Prudent Parent Response (up to 30 minutes)

Before activating the full missing youth protocol, staff take the following prudent parent steps simultaneously:

- Attempt direct contact: call the youth's cell phone; send a text message; try any messaging app they use
- Contact anyone who may know: friends named in the ISP; family members on the contact list; school contacts if a school day
- Search the facility and grounds thoroughly
- Check known community locations the youth frequents
- Notify the on-call supervisor immediately and begin the chronological missing youth log

If at any point there is reason to believe the youth's safety is in immediate danger, skip Phase 1 and call 911 immediately.

18.3 Phase 2 — Missing Youth Confirmed

Action	Required By Whom / When
Notify MCFD social worker or PCS (1-800-663-9122)	Shift lead; immediately upon confirming missing
Call 911 / RCMP if: immediate safety concern; social worker directs police; or no social worker direction after 1 hour	Shift lead
Continue chronological log: all contacts attempted, responses received, times	Both staff on shift
Notify Program Manager	Shift lead; within 30 minutes of confirming missing
Prepare youth's description and recent photo for police if needed	Shift lead
Notify Executive Director	Program Manager; same day for any missing youth over 2 hours
MCFD Critical Incident Report	Program Manager; within 24 hours regardless of outcome

18.4 Information for Police / RCMP

- Legal name (and any known aliases or preferred name)
- Date of birth and age
- Physical description: height, weight, hair colour and length, eye colour, build, distinguishing features
- Last known clothing in full detail
- Cell phone number and any other contact information
- Time and location last seen; who last saw them and what was said
- Known friends, associates, or locations they may go
- Risk factors: suicidal ideation history; substance use; self-harm; known unsafe persons; trafficking or exploitation concerns

⚠ CRITICAL: Do NOT disclose to police, media, or any member of the public that the youth is in government care under the CFCSA. This is a privacy breach under FOIPPA. State only that they are a resident of your program.

18.5 When the Youth Returns

- Conduct a physical and emotional wellness check immediately — warm, non-punitive, and without interrogation
- Offer food, warmth, and a calm space to settle before any structured conversation
- Notify police (if involved), MCFD social worker, and Program Manager immediately
- Document the return in the missing youth log: time, condition, any disclosures
- Schedule a debriefing with the youth within 24 hours when they are ready
- Review the ISP and safety plan within 5 business days of any missing episode
- Critical Incident Report finalized with return information added

19. PHYSICAL RESTRAINT POLICY AND POST-INCIDENT RESPONSE

Regulatory Reference: MCFD Standards C.3, C.6; MCFD Policy 5.13 (Restraint and Seclusion); CFCSA s.70; OHS Reg. s.4.27–4.31

19.1 Position Statement

⚠ Physical restraint is a measure of last resort used ONLY to prevent immediate risk of serious physical harm to the youth, another person, or staff. It is never used as a consequence, punishment, or for staff convenience. Every use of physical restraint is a Critical Incident requiring immediate MCFD notification.

19.2 Authorization Criteria — All Must Be Met

Physical restraint is authorized only when ALL of the following conditions are simultaneously met:

- There is an imminent risk of serious physical harm to the youth, another youth, or a staff member — not merely disruptive behaviour
- Less restrictive means have been tried and have failed, or the immediacy of the harm makes waiting for them to work unsafe
- The staff member initiating restraint is currently certified in the organization's approved physical intervention program (NVCi or equivalent)
- The restraint uses the minimum force necessary for the minimum time necessary
- A second staff member is present or has been called

19.3 Approved Techniques

Only holds and techniques covered in the current Non-Violent Crisis Intervention (NVCi) curriculum — or equivalent approved by the Program Manager — may be used. Staff may not improvise, apply unapproved holds, or combine approved holds in ways not covered in their training.

19.4 Absolutely Prohibited Practices

Prohibited Practice	Reason and Authority
Prone (face-down) restraint	Asphyxiation risk; prohibited by MCFD Policy 5.13
Any hold restricting breathing or blood flow to the brain	Life-threatening; prohibited by MCFD Policy 5.13 and OHS Reg.
Pressure on the neck, throat, chest, or abdomen	Life-threatening; absolutely prohibited
Seclusion: placing a youth in a space they cannot leave	Prohibited by MCFD Standards C.3 and CFCSA s.70
Restraint used as a consequence, punishment, or deterrent	Violates CFCSA s.70 and MCFD Standards C.3
Restraint by a non-NVCi-certified staff member	Except where a certified staff member is not present and life safety requires action until help arrives
Leaving a restrained youth	Prohibited by MCFD Standards C.3; life safety obligation

unattended

19.5 During Restraint — Required Practices

- Continuous verbal communication with the youth: calm, non-threatening, focused on safety ('I'm going to help you be safe')
- Continuous monitoring for signs of distress: skin colour, breathing rate and quality, consciousness, complaint of pain
- Release immediately when the immediate risk has passed — restraint does not continue as a consequence
- The non-restraining staff member monitors the youth's condition and is the designated caller if 911 is needed

19.6 Mandatory Post-Restraint Actions — All Required

Action	Timeline
Medical wellness check of the youth	Immediately on release
Call 911 for any injury, pain complaint, breathing concern, or loss of consciousness	Immediately
Notify on-call supervisor	Immediately; do not wait until end of shift
Debrief with the youth when regulated	Same shift if possible; within 24 hours at latest
Debrief all involved staff	Before end of shift
Notify MCFD social worker or PCS	Within 2 hours
Offer EFAP to all involved staff	Before end of shift
Complete Critical Incident Report	Within 2 hours of the incident
JHSC worker safety notification	Within 24 hours
ISP and BSP review	Within 5 business days

19.7 Restraint Trend Monitoring

The Program Manager reviews all restraint incidents monthly. Three or more restraint incidents involving the same youth within 30 days triggers an urgent ISP review, additional clinical consultation, and an enhanced BSP. The JHSC reviews restraint incidents from an occupational health and safety perspective. Persistent restraint use may indicate a placement that is not meeting the youth's needs, which is discussed with MCFD.

20. YOUTH SAFETY: ENVIRONMENTAL RISK MANAGEMENT

Regulatory Reference: MCFD Standards F.1, F.4, C.1; RCR s.22–56; OHS Reg. Part 4

20.1 Environmental Risk Assessment

Every Evergreen Community Care facility undergoes a formal Environmental Risk Assessment at: initial program set-up; annually; when a new youth with identified risk factors is admitted; and following any incident that reveals an uncontrolled environmental hazard.

Risk Category	Assessment Elements
Ligature and self-harm risks	Anchor points in bedrooms, bathrooms, closets, and common areas; access to sharps, chemicals, medications, cords, strings, and other ligature materials; window opening restrictions
Elopement risk	Door and window locking systems; perimeter security; exterior lighting; sight-lines from staff areas to exits; alarm systems
Water safety	Hot water temperature at all taps (maximum 49°C, RCR April 2026); bath and shower water temperature checking procedure; outdoor water bodies
Fire risks	Cooking supervision; ignition sources (lighters, matches, candles) accessible to youth with fire-setting history; fire detection coverage
Sharp objects	Kitchen implements; craft supplies; tools; broken items accessible to youth
Chemicals and medications	Storage security; label integrity; quantities stored
Outdoor environment	Perimeter security; neighbourhood hazards; traffic; community risks
Youth-specific hazards	Any hazard specific to a youth's individual history, behaviour, or risk profile identified in their Safety Plan

20.2 Individualized Safety Adaptations

When a youth with specific identified risks is admitted, the Environmental Risk Assessment is reviewed through the lens of that youth's specific risk profile. Required safety adaptations are documented in the ISP Safety Plan and implemented within 24 hours of admission. Adaptations are reviewed at every ISP review.

20.3 Shift Safety Check

At the start of every shift, the shift lead completes a brief safety walkthrough:

- All exterior doors and windows are in their expected security state
- Medication cabinet is locked; controlled substance count matches the log
- Chemical storage is locked and secure
- Kitchen implements are appropriately secured per any individual youth's safety plan
- No new hazards have been introduced (e.g., tool left out from maintenance work; new cleaning product not yet locked)
- All youth are located and their status is as expected per the handover

The shift safety check is documented in the shift log.

21. EMERGENCY PREPAREDNESS FOR RESIDENTIAL CARE

Regulatory Reference: MCFD Standards F.4; OHS Reg. Part 4; BC Fire Code; Emergency Program Act

See also: OHS Program Binder (ECC-OHS-POL-2026-002), Sections 12 and 27, for the organization-wide emergency preparedness program. This section addresses residential care-specific requirements and modifications.

21.1 Emergency Procedures Specific to Residential Care

Emergency	Residential Care Modifications and Procedures
Fire evacuation	Youth are evacuated in the same manner as family members; staff ensure all youth exit before leaving. Youth who resist evacuation: verbal guidance first; physical guidance (not restraint) if immediate danger. Youth who cannot self-evacuate: Area of Rescue Assistance identified and communicated to fire services. Evacuation plan accounts for all youth and staff by name.
Medical emergency involving a youth	911 called immediately; a staff member accompanies the youth to hospital when possible; MCFD social worker notified within 1 hour; family notified per care plan. One staff member remains with the other youth in the home.
Youth-initiated violence (assault on staff or other youth)	Other youth removed from the immediate area first; de-escalation attempted; backup called; 911 called if situation cannot be safely managed with available staff; MCFD notified same day.
Security threat (unknown person, break-in, external threat)	Lock all exterior doors; secure youth in interior areas away from threat; call 911; do not open door until police confirm all-clear; MCFD notified same day.
Missing youth during an emergency	Location communicated immediately to fire/police services; staff do not re-enter an unsafe building to search; MCFD notified.
Power outage (extended)	Flashlights and emergency lighting activated; Program Manager notified; temperature in the home monitored; refrigerated medications managed per temperature log protocol; MCFD notified if outage exceeds 4 hours.

21.2 Emergency Drill Requirements

Drill Type	Frequency	Documentation
Fire evacuation (announced)	Quarterly	Date, time, duration, all occupants listed, deficiencies noted
Fire evacuation (unannounced, night/weekend)	Annually	Same as above; program Manager reviews
Medical emergency tabletop (team meeting)	Annually	Meeting minutes; scenario and decisions documented

Missing youth protocol review	Annually; and after any missing youth event	Meeting minutes
Lockdown / security threat tabletop	Annually	Meeting minutes

21.3 Emergency Contact Information Posted in Every Facility

Emergency contacts are posted at every telephone and in each youth's room:

- 911 (police, fire, ambulance)
- MCFD Provincial Centralized Screening: 1-800-663-9122
- On-call supervisor: [posted with current number]
- Program Manager: [posted with current number]
- Poison Control: 1-800-567-8911
- Crisis Centre BC: 1-800-784-2433
- Kids Help Phone: 1-800-668-6868
- Advocate for Children and Youth: 1-800-476-3933
- Facility address: [posted at every telephone for emergency callers]

PART V — CULTURAL SAFETY AND EQUITY

22. INDIGENOUS RECONCILIATION FRAMEWORK AND CULTURAL SAFETY PLAN

Regulatory Reference: DRIPA (SBC 2019, c.44); CFCSA s.2, 70, 71; TRC Calls to Action; UNDRIP Articles 7, 8, 14, 21; MCFD Standards G.3

22.1 Organizational Commitment

Evergreen Community Care acknowledges with humility and accountability that Indigenous children are vastly overrepresented in government care — a direct consequence of colonization, the residential school system, the Sixties Scoop, and systemic racism. We are committed to being part of the solution, not the continuation of the problem.

Our commitment is guided by the Declaration on the Rights of Indigenous Peoples Act (DRIPA), the Truth and Reconciliation Commission's Calls to Action (particularly Call to Action #65 regarding Indigenous child welfare), and UNDRIP. This commitment requires ongoing action, not just stated values.

22.2 Placement Principles for Indigenous Children and Youth

Where residential care placement is required for an Indigenous child or youth, the following placement preference order applies and is actively pursued before accepting a group care placement:

Priority Level	Placement Type
First priority	Extended family of the child — Indigenous family members
Second priority	Extended family or persons nominated by the child's family
Third priority	A family approved by the child's Indigenous community or Nation
Fourth priority	Another Indigenous family
Fifth priority	A non-Indigenous family committed to actively supporting the child's cultural identity
Group care (such as Evergreen)	Only when family-based options have been genuinely explored and are not currently available or appropriate

When Evergreen Community Care accepts an Indigenous youth into group care, it does so with the explicit understanding that cultural connection, identity support, and Nation involvement are non-negotiable components of the care provided.

22.3 Cultural Plan Requirements

Every Indigenous child and youth has a written Cultural Plan, developed within 30 days of placement, in genuine collaboration with the youth, their family, and their Nation. The Cultural Plan (Appendix H) is a living document updated at every ISP review.

The Cultural Plan includes:

- The youth's specific Nation, band, or community affiliation and geographic territory
- The youth's current relationship with their cultural identity, language, and community
- The youth's own expressed goals for cultural connection

- Specific cultural activities, ceremonies, events, and practices to be supported
- Elders, cultural mentors, Knowledge Keepers, and DAA representatives involved
- Traditional foods, medicines, and healing practices to be accommodated in the home
- Language learning opportunities where the youth is interested
- Responsibilities of Evergreen Community Care, MCFD, and the Nation

22.4 Nation and Community Notification

- Evergreen Community Care supports MCFD's obligation to notify the relevant Indigenous community or Nation prior to or upon placement of an Indigenous child
- Band social workers, DAA representatives, and community liaisons are welcomed at care team meetings
- Nation representatives are provided with information about the youth's progress and cultural activities, consistent with the youth's consent and care plan
- Any disagreement between Evergreen Community Care's practices and a Nation's guidance on cultural matters is escalated to the MCFD social worker and Program Manager for resolution

22.5 Staff Cultural Safety Obligations

- All staff complete Indigenous Cultural Safety and Humility training (FNHA-recognized or equivalent) within 60 days of hire and annually thereafter
- Smudging, ceremony, drumming, and other cultural practices are accommodated in the home without judgment or restriction
- Sacred items and medicines are treated with respect; staff who are unsure ask the youth or their cultural supports
- Cultural foods are incorporated into meal planning as requested by Indigenous youth; the program budget accommodates this
- Staff examine their own assumptions and biases related to Indigenous peoples through training, supervision, and reflection; cultural humility is ongoing, not achieved
- Any racially discriminatory comment or action by any staff member, volunteer, or other youth is addressed immediately and documented; MCFD is notified

22.6 Indigenous Data and Identity

A youth's Indigenous status, Nation affiliation, and cultural identity are treated with respect and appropriate privacy:

- Indigenous status is documented in the ISR with the youth's knowledge; it is shared with staff on a need-to-know basis for care purposes
- Staff do not make assumptions about a youth's Indigenous identity based on appearance or name
- Information about a youth's cultural identity and practices is shared with the youth's Nation and MCFD in accordance with the care plan; not disclosed to others without authorization

23. GENDER-AFFIRMING CARE PROGRAM

Regulatory Reference: MCFD Standards C.1.1(b), D.1.5(b), D.1.13(b), D.6(g), G.3.2(b), G.4.3(c) (amended April 2024); BC Human Rights Code; CFCSA s.70

23.1 MCFD Standard and Organizational Commitment

Gender-affirming care is defined by MCFD as: the processes through which caregivers, staff, services, and organizations care for and support an individual, while recognizing and acknowledging their gender identity and expression to promote their safety, well-being, and belonging. This is a mandatory MCFD Standard, not an optional program element.

23.2 Core Obligations — All Staff

Obligation	Detail
Use correct name and pronouns	Every staff member uses each youth's stated name and pronouns consistently and immediately; 'deadnaming' or deliberate misgendering is a Code of Ethics violation subject to disciplinary action
Affirm gender identity and expression	Youth's gender identity and expression is supported, affirmed, and nurtured in daily care; staff do not question, pathologize, or express discomfort with any gender identity
Maintain confidentiality	A youth's gender identity is personal and confidential; it is not shared with family, MCFD (beyond care team), or others without the youth's informed consent unless a documented safety reason requires it
Training	All staff complete gender-affirming care training within 60 days of hire and annually thereafter; training covers: terminology; the health impacts of non-affirmation; practical application in daily care; and confidentiality
Respond to discrimination	Any gender-based harassment, misgendering, or discrimination from staff, other youth, or visitors is addressed immediately and documented as an incident
Inclusive environment	The physical environment is welcoming to youth of all gender identities; gendered language is minimized in signage, programming, and daily interaction

23.3 Environment and Practical Accommodations

- Bedroom and bathroom arrangements reflect each youth's gender identity wherever operationally feasible and with the youth's input into the decision-making process
- Youth have access to gender-affirming clothing, personal care items (including skin care, make-up, hair care products), and hygiene supplies that reflect their gender identity
- Youth are not restricted in their clothing choices based on gender stereotypes; their clothing expression is supported
- Youth have access to gender-affirming mental health resources and community organizations serving LGBTQ2S+ youth

- If a youth is in the process of gender transition (social or medical), the Program Manager consults with the MCFD social worker and, where relevant, the youth's healthcare providers to ensure appropriate support is in place

23.4 Care Plan Integration

Where a youth has identified gender-affirming care needs, these are documented in the ISP with specific supports, accommodations, and review timelines. The ISP is updated whenever the youth's gender expression or care needs evolve.

24. ANTI-RACISM AND EQUITY PRACTICE

Regulatory Reference: BC Human Rights Code; CFCSA s.2; DRIPA; MCFD Standards

24.1 Commitment

Evergreen Community Care is committed to an actively anti-racist organizational culture and practice. We recognize that racism is not merely individual prejudice — it is also systemic, structural, and embedded in practices that can cause harm even without conscious intent. We commit to examining and changing both individual practices and organizational structures that perpetuate inequity.

24.2 Staff Responsibilities

- Model respectful, anti-racist behaviour in all interactions with youth, families, colleagues, and community partners
- Respond immediately and directly to any racist comment, joke, or behaviour from any person in the program environment — including from other youth. Silence in the presence of racism is not neutral.
- Examine personal biases through training, supervision, and ongoing reflection; cultural humility is a career-long practice, not a one-time training event
- Document and report experiences of racism in the program environment — whether directed at youth, staff, or families — through the incident reporting process
- Report experiences of racism or racial discrimination by other staff to the Program Manager; where the Program Manager is the subject, to the Executive Director

24.3 Organizational Anti-Racism Actions

- Annual review of program practices for disparate impact on racialized youth (e.g., whether consequences are applied equally; whether programming reflects the cultures of youth served)
- Active effort to recruit and retain staff from Indigenous, racialized, and equity-seeking communities
- Physical environment that reflects the cultural diversity of youth served: artwork, images, books, and media that represent multiple cultures
- Meals and programming that include cultural foods, traditions, and celebrations representing the youth in the home
- Training: all staff complete anti-racism and equity training within 60 days of hire and at least every 2 years thereafter

24.4 Responding to Racism Directed at Youth

When a youth reports experiencing racism — from staff, other youth, family members, or community members — Evergreen Community Care:

- Believes the youth's account and responds with immediate validation and support
- Documents the incident and reports it as a Critical Incident to the MCFD social worker within 24 hours
- Takes immediate corrective action where the racism is from a staff member or volunteer
- Supports the youth to access advocacy and additional support through the MCFD social worker, Advocate, or cultural supports
- Reviews whether the incident reflects a systemic issue requiring organizational response

25. TRAUMA-INFORMED CARE FRAMEWORK

Regulatory Reference: MCFD Standards D.1, G.3; BC Trauma-Informed Practice Guide (2019); SAMHSA TTIC Framework

25.1 Definition

Trauma-informed care is not a single intervention or treatment modality. It is an organizational and practice framework that recognizes the widespread impact of trauma, integrates knowledge about trauma into policies and practices, and seeks to avoid re-traumatization while actively supporting healing. At Evergreen Community Care, trauma-informed care is how we operate, not something we do in addition to operating.

25.2 Core Principles — SAMHSA's Six Foundations

Foundation	Application at Evergreen Community Care
Safety	Both physical and psychological safety for every youth and staff member; predictable routines; transparent communication; no surprises in consequences
Trustworthiness and transparency	Staff are honest, even when the truth is difficult; promises are kept; decisions are explained; youth are informed of what is happening and why
Peer support	Youth are supported to connect with and support one another; peer relationships are recognized as therapeutic resources
Collaboration and mutuality	Care is done with youth, not to them; power differentials are acknowledged and minimized; staff share power with youth through choice and voice
Empowerment and choice	Youth have choices and a voice in every aspect of their care; their strengths are recognized and built upon; they develop mastery and efficacy through graduated responsibility
Cultural, historical, and gender responsiveness	The organization recognizes the relationship between culture, identity, and trauma; cultural safety and gender affirmation are components of trauma-informed care

25.3 Trauma-Informed Daily Practice

Trauma-informed care is evident in how staff interact with youth every day:

Practice Area	Trauma-Informed Approach
Responding to behaviour	Ask 'What happened to you?' not 'What's wrong with you?'; recognize escalation as a stress response, not a choice
Maintaining consistency	Predictable schedules, expectations, and responses; same rules, same warmth, every day, every staff member
Setting limits	Clear, calm, non-punitive limit-setting that maintains the relationship; avoid power struggles
Welcoming and transitions	Arrival and departure moments are transitions that can trigger trauma

	responses; they are managed with extra care and attention
Physical environment	Comfortable, home-like spaces with minimal institutional features; youth-chosen art and decor; privacy; calm colours and sounds
Language	Avoid pathologizing language ('manipulative', 'attention-seeking', 'defiant'); use descriptive, strengths-based language
Sensory needs	Awareness of sensory triggers; accommodation of sensory sensitivities in the environment and routine

25.4 Secondary Traumatic Stress and Staff Wellbeing

Staff who provide care to traumatized youth are themselves at significant risk of secondary traumatic stress and compassion fatigue. These are recognized occupational hazards, not signs of weakness. Evergreen Community Care addresses this through organizational support:

- Reflective supervision is provided to all direct care staff at minimum monthly; it is structured to create space for emotional processing, not just task review
- Team debriefs are offered following every significant critical incident before the end of the shift
- EFAP is actively promoted and accessible from the first day of employment; the number is posted in every staff area
- Scheduling and workload decisions take emotional and clinical load into account, not only hours worked
- A culture in which asking for help, admitting difficulty, and seeking support is valued and never penalized

PART VI — STAFFING AND PROGRAM GOVERNANCE

26. STAFFING RATIOS AND DEPLOYMENT

Regulatory Reference: RCR s.66–72; MCFD Standards G.2; Contracted Service Agreement; OHS Reg. s.4.21

26.1 Minimum Staffing Standards

Staffing ratios are governed by the Residential Care Regulation, the facility licence, and the Contracted Service Agreement. Where these specify different minimums, the highest standard prevails. Evergreen Community Care maintains ratios at or above the following minimums at all times:

Time Period	Minimum Staffing Requirement	Enhanced Requirement Triggers
Daytime shift (typically 07:00–23:00)	Per licence and Contracted Service Agreement; minimum 1:3 for a typical 3-youth home	Any youth with a current active safety plan requiring enhanced supervision; two or more high-acuity youth in the home simultaneously
Overnight shift (typically 23:00–07:00)	Minimum 1 awake staff on duty; no youth is ever left without a present, awake staff member	Youth with a documented self-harm or elopement risk requiring enhanced overnight supervision; acute mental health crisis
Community outings	Per Appendix K outing safety plan; minimum 1 staff per 2 youth for standard community activities	1:1 for youth with active self-harm or elopement risk during outings; 2 staff minimum for outings involving swimming or significant safety hazards
Medical emergencies	Minimum 1 staff accompanies youth to hospital	Always; a second staff member remains with other youth at the facility

26.2 When Ratios Cannot Be Maintained

If staffing falls below minimums due to an unexpected vacancy, illness, or emergency:

- The shift lead notifies the Program Manager immediately
- The Program Manager takes all reasonable steps to arrange additional coverage within 2 hours
- If minimum ratios cannot be restored: MCFD social worker is notified; a Critical Incident Report is completed; the Program Manager documents all steps taken to address the shortfall
- Staff are never pressured to work in unsafe conditions solely to avoid the inconvenience of an organizational staffing problem

26.3 Shift Handover Requirements

The shift handover is a critical safety moment. Minimum handover requirements:

- Minimum 15-minute in-person overlap between outgoing and incoming shift lead

- Verbal handover covers: location and status of every youth; mood and presentation of each youth; any incidents or concerns from the shift; medications administered and any exceptions; any outstanding tasks; controlled substance count reconciled
- Written handover: shift log completed before outgoing staff depart
- Incoming shift lead confirms receipt of all critical information; any unclear information is clarified before the outgoing shift departs

26.4 On-Call Supervisor Availability

An on-call supervisor with authority to make operational decisions is available to all on-duty staff at all times, 24 hours a day, 7 days a week:

- On-call schedule is posted one week in advance
- On-call supervisor responds to calls within 15 minutes
- On-call supervisor can authorize: additional staffing; emergency purchases; MCFD after-hours reporting; crisis responses
- Gaps in on-call coverage are treated as a safety concern and escalated to the Program Manager

27. STAFF ORIENTATION AND TRAINING PROGRAM

Regulatory Reference: MCFD Standards G.3; HR Policy Binder Section 5; OHS Reg. s.3.22–3.23

27.1 New Staff Orientation — Before First Independent Shift

Every new staff member completes a structured orientation before working independently with youth. 'Working independently' means without a senior staff member present and available to coach. The following topics are covered:

Topic	Content	Format
Legislative framework	CFCSA (rights, best interests, duty to report), CCALA, RCR, DRIPA — what they mean in daily practice	Reading + verbal review with supervisor
Youth rights (CFCSA s.70)	Every right; what they mean; staff obligations; how to uphold them	Reading + quiz/discussion
Mandatory reporting (s.14 CFCSA)	Personal, non-delegable duty; what triggers it; how to report; what to document	Scenario-based discussion
Code of Ethics and professional boundaries	Complete Code; prohibited practices; dual relationships; social media	Reading + signed acknowledgment
Behaviour management	Positive approaches; prohibited practices; NVCI overview; BSP structure	Reading + role play with supervisor
Critical Incident reporting	Categories; timelines; how to write a CI report	Reading + sample report review
Medication administration	Overview; five rights; MAR; errors — full competency training before first medication duty	Supervised practice + competency sign-off
Emergency procedures	Fire evacuation; missing youth; medical emergency; lockdown	Walkthrough of facility; procedures reviewed
Trauma-informed care	Foundations; daily practice; de-escalation; secondary trauma	Reading + group discussion
Cultural safety	Indigenous reconciliation; cultural safety practices; Cultural Plans	Reading + discussion with supervisor
Gender-affirming care	Terminology; obligations; practical applications	Training module
OHS orientation	Worker rights; hazards; incident reporting; PPE; working alone procedure	OHS Program Binder orientation

27.2 Mandatory Training Matrix

All training requirements are tracked in the Staff Training Compliance Matrix (Appendix L), updated monthly by the Program Manager:

Training	Audience	Timeline	Renewal
NVCI or equivalent	All direct care staff	Within 90 days; module 1 before solo shifts	Annual
Indigenous Cultural Safety and Humility	All staff	Within 60 days	Annual
Gender-Affirming Care	All staff	Within 60 days	Annual
Trauma-Informed Care foundations	All direct care staff	Within 60 days	Every 2 years
Suicide and self-harm response (ASIST or equiv.)	All direct care staff	Within 60 days	Annual
Mandatory Reporting (CFCSA s.14)	All staff	Before first independent shift	Every 2 years
Medication Administration	Authorized staff only	Before first medication duty	Annual competency
First Aid / CPR Level C	All direct care staff	Within 30 days	Annual
OFA Level 2 (designated attendant per shift)	Minimum 1 per shift	Before duty as designated attendant	Per certification cycle
WHMIS 2015	All staff	Before first shift	Every 3 years
Workplace Violence Prevention	All staff	Before first shift	Annual
Driver Safety (with abstract)	Any driver of youth	Before first driving duty	Annual abstract review
Working Alone	Staff on solo overnight	Before first solo overnight	Annual

27.3 Training Records and Compliance

- Certificates and completion records for every training are retained in each staff member's personnel file
- The Program Manager reviews training compliance monthly; staff with overdue mandatory training are not assigned duties requiring that training until compliance is restored
- Training gaps identified in MCFD audits or licensing inspections are addressed within 30 days with a documented remediation plan
- Training costs are borne by the organization; staff are compensated at their regular rate for training time

28. SUPERVISION AND REFLECTIVE PRACTICE

Regulatory Reference: MCFD Standards G.4; HR Policy Binder Section 7; best practice in child and youth care

28.1 Individual Supervision Structure

Individual supervision is a professional obligation and a clinical tool, not a management check-in. Effective supervision in child and youth care requires both accountability and reflective practice.

Supervision Element	Requirement
Frequency	All direct care staff: minimum monthly. New staff (first 90 days) and staff on performance improvement plans: minimum every 2 weeks. Program Managers receive supervision from the Executive Director minimum monthly.
Duration	Minimum 60 minutes per session; uninterrupted
Format	In-person where possible; scheduled in advance with protected time
Location	Private; confidential
Documentation	Brief supervision notes retained in the personnel file; accessible to the supervisor and staff member only
Content	See required topics below

28.2 Required Supervision Content

Content Area	What Is Explored
Emotional check-in	How is the staff member doing? What is affecting them at work? Signs of secondary traumatic stress or compassion fatigue?
Case review	Specific youth discussed: what is going well; what is challenging; what approaches are working and not working; BSP and ISP currency
Practice reflection	A recent significant interaction examined: what was the staff member's internal experience? What informed their response? What might they do differently?
Critical incidents	Any incidents since the last supervision: what happened; was the response appropriate; what does the staff member need to process or learn?
Professional development	What is the staff member working on professionally? What support do they need?
Performance feedback	Honest, specific, two-way; includes recognition of strengths and areas for growth
Wellbeing	Physical and psychological health; workload; work-life balance; EFAP if warranted

28.3 Team Meetings and Group Supervision

- Full team meetings are held monthly at minimum; attendance is part of the role, not optional
- Agenda includes: case review (anonymized); critical incident learning; practice discussion or training; team wellbeing; organizational updates
- A monthly team case consultation — reflective group discussion of one or more youth situations — is a permanent component of team meetings
- Staff are encouraged to raise practice dilemmas, ethical concerns, and system frustrations in team meetings; the forum is psychologically safe and non-punitive

28.4 Critical Incident Debrief

Following every significant critical incident, all staff involved receive a structured debrief before the end of the shift:

- Facilitated by the Program Manager or a trained CISM debriefer
- Not a disciplinary process; focused solely on wellbeing and learning
- Covers: what happened; how staff are feeling; what went well; what was hard; what support is needed
- Followed by individual EFAP referral for any staff member who requests additional support or appears significantly distressed

29. VOLUNTEER, STUDENT, AND CONTRACTOR MANAGEMENT

Regulatory Reference: MCFD Standards G.1, H.1; Criminal Records Review Act; OHS Reg. s.3.20–3.21

29.1 Screening Requirements — All Persons with Youth Access

Screening Requirement	Who It Applies To	Before What Activity
Vulnerable Sector Criminal Record Check (CRCA)	All volunteers, students, and contractors with direct or unsupervised youth access	Before any access to youth
Reference checks (minimum 2)	All volunteers and students with ongoing roles	Before placement begins
Orientation to rights, prohibited practices, and confidentiality	All volunteers, students, and contractors	Before first youth contact
Signed Code of Ethics and Confidentiality Agreement	All volunteers and students	Before first youth contact
Review of specific youth's ISP highlights (where relevant to the role)	Volunteers and students with ongoing direct care involvement	Before working with that youth

29.2 Practicum Student Supervision

Practicum students from Child and Youth Care, Social Work, Psychology, and related programs are welcome at Evergreen Community Care:

- Each student has a designated field supervisor — a senior staff member who is responsible for the student's orientation, supervision, and evaluation
- Students are never left alone as the sole staff member responsible for youth, regardless of their level of training
- Students' activities are commensurate with their training level; new students observe and co-work before taking a lead role in any interaction
- Students are included in team meetings and supervision; they are supported to ask questions and reflect on practice
- Concerns about a student's practice or professionalism are addressed through both the organizational supervisor and the academic institution's field liaison

29.3 Volunteer Program

Volunteers enrich youth's lives through relationships, skills, and community connection. Volunteer roles are clearly defined, supervised, and appropriate to the program:

- Volunteer roles are designed to supplement — not replace — direct care staff

- Volunteers work alongside staff; they are never assigned to be alone with youth without a staff member present
- Volunteers receive the same rights and prohibited practices orientation as new staff
- A volunteer who witnesses a rights violation, discloses concerns about a youth's safety, or observes prohibited practices is directed to report to the Program Manager immediately

29.4 Contractor Safety

Contractors (maintenance, trades, specialized services) working in Evergreen Community Care facilities:

- Complete visitor sign-in and are escorted by a staff member while youth are present
- Are not left alone with youth under any circumstances
- Receive a brief facility safety orientation from the admitting staff member: emergency exits; who to contact in an emergency
- WorkSafeBC registration is confirmed for all contractors performing physical work on premises

30. PROGRAM REVIEW AND QUALITY ASSURANCE

Regulatory Reference: MCFD Standards H.2; Contracted Service Agreement; QA Program Binder (ECC-QA-2026-001)

30.1 QA Program Overview

Evergreen Community Care's Quality Assurance program is described in full in the QA Program Binder (ECC-QA-2026-001). This section provides the integration between this Residential Care Program Manual and the QA Program.

Quality assurance at Evergreen Community Care answers four questions: Are we safe? Are we compliant? Are we improving? Are we listening? Every QA activity is designed to answer at least one of these questions.

30.2 QA Activity Calendar

Frequency	QA Activity	Responsible
Each shift	Shift safety check; shift log completed; medication count reconciled	Shift lead
Weekly	Shift log review (spot sample) by supervisor; any concerns documented	Program Manager or designate
Monthly	Facility safety inspection (Appendix J); training compliance review; CI incident trend review; on-call supervision	Program Manager / JHSC
Quarterly	CI trend analysis report; full ISP file audit (25% sample); KPI dashboard reviewed	Program Manager
Biannually	Full ISR file audit; Youth and Family Feedback Survey	Program Manager
Annually	MCFD Standards A-H self-assessment; Staff Satisfaction Survey; Annual Program Review; full manual review and update	Program Manager + Executive Director
Annually (external)	Licensing inspection by Health Authority; MCFD contract monitoring visit; OHS external review	External parties

30.3 Key Performance Indicators

KPI	Target
Critical Incidents reported to MCFD within required timeline	100%
ISP reviews completed on schedule	100%
Staff with all mandatory training current	100%

Physical restraint incidents	Year-over-year reduction; zero tolerance for any prohibited technique
Youth school attendance rate (enrolled youth)	85%+ attendance; 100% enrolled in appropriate educational program
Youth satisfaction rating (annual survey)	70%+ 'satisfied' or 'very satisfied'
Family satisfaction rating (annual survey)	70%+ 'satisfied' or 'very satisfied'
Staff retention rate (beyond probation)	75%+ annually
Occupancy rate	Per Contracted Service Agreement targets
Placement stability (youth remaining in placement beyond 90 days)	75%+ placements stable beyond 90 days
Licensing deficiencies	Zero critical deficiencies; all lower-level deficiencies remediated within required timelines

30.4 Policy Review and Approval

This Residential Care Program Manual is reviewed annually each July, and also upon:

- Amendment to MCFD Standards, the CFCSA, CCALA, Residential Care Regulation, or DRIPA
- MCFD monitoring visit or licensing inspection with recommendations
- Critical incident revealing a policy gap or compliance issue
- Significant change to the program's population, capacity, or service model

30.5 Version History

Version	Date	Summary of Changes
1.0	January 2025	Initial policy manual; 15 sections covering MCFD Standards A–H and basic supplementary policies
2.0	July 1, 2026	Comprehensive revision: full MCFD Standards alignment to April 2026 amendments including gender-affirming care (April 2024), water/beverage temperature standards (April 2026), missing child protocol updates (May 2025)
3.0	July 1, 2026	Complete Comprehensive Program Manual: 30 sections + 12 appendices; added Parts III–VI covering all operational practice areas, full safety protocols, cultural safety framework, equity practice, trauma-informed care, staffing, supervision, and QA; all MCFD Standards fully operationalized

SIGNATURES AND APPROVAL

This Residential Care Program Manual has been developed and approved in good faith to meet or exceed all current MCFD residential care standards, licensing requirements, and contracted service obligations of Evergreen Community Care Inc.

Signatory	Signature / Date
Executive Director	Signature: _____ Date: _____
Program Manager, Residential Care	Signature: _____ Date: _____
Board Chair or Delegate	Signature: _____ Date: _____
MCFD Contract Manager (Acknowledgment)	Signature: _____ Date: _____

APPENDIX A — ADMISSION AND INTAKE CHECKLIST

Complete at admission. File original in Individual Service Record. Retain for minimum 7 years.

Field	
Youth Legal Name	
Preferred Name / Pronouns	
Date of Birth	
Date of Admission	
Legal Status (e.g., s.35, Continuing Custody Order)	
MCFD Social Worker (Name and Phone)	
Referring Social Worker (if different)	
Admitting Program Manager	
Admitting Staff Member	

Task	Done?	Notes / Action Required
MCFD placement authorization confirmed	☐	
Placement decision documented by Program Manager	☐	
Facility capacity confirmed	☐	
Youth welcomed; bedroom assigned and shown	☐	
Personal belongings inventoried and stored	☐	
Rights information reviewed verbally + Appendix C provided	☐	
Advocate contact number given: 1-	☐	

800-476-3933		
Complaint process explained	☐	
Emergency contact information for social worker confirmed	☐	
Medical information reviewed; allergies and health alerts confirmed	☐	
All medications received; MAR initiated; locked	☐	
Controlled substance count done with incoming and outgoing staff	☐	
Hot water temperature at all taps confirmed not exceeding 49°C	☐	
Court orders and access restrictions reviewed	☐	
Environmental risk assessment reviewed for this youth's profile	☐	
Safety adaptations required? If yes — documented in preliminary plan	☐	
Indigenous status confirmed; Cultural Plan meeting scheduled within 30 days	☐	
School enrollment status confirmed; enrollment arranged if needed	☐	
Preliminary ISP / safety plan created within 48 hours	☐	
Full ISP meeting scheduled within 14	☐	

days		
Youth orientation (Appendix B) completed within 48 hours	☐	

Program Manager Signature / Date	
Admitting Staff Signature / Date	

APPENDIX B — NEW YOUTH ORIENTATION GUIDE

Staff guide for welcoming a new young person. Use as a conversation guide, not a script. Go at the youth's pace.

Welcome Statement

'Welcome to [program name]. My name is [staff name] and I'll be working with you. I want to show you around and tell you how things work here. There are no stupid questions — ask me anything.'

The Tour

Walk the youth through the home at their pace. Show them:

- Their bedroom — it is their space; staff knock before entering
- Where their belongings will be stored; their personal locked storage
- Bathroom(s)
- Kitchen and when meals are served
- Common areas — TV, games, outdoor space
- All exit doors and the assembly point
- Where the fire extinguisher and first aid kit are
- Emergency contacts poster — point to it and name what each number is for
- Staff office or workspace

Daily Routine Overview

Cover: wake-up and bedtime routines; meal times; school schedule; chores; activity and free time; who they will see on different shifts.

Rights Review

'You have rights in this home. Important ones. The handout I gave you lists them. The most important ones to know right now are:'

- You can always contact your social worker, the Advocate (1-800-476-3933), or the police — in private, without anyone's permission.
- No one can hit you or physically punish you for any reason.
- You can make a complaint if something is wrong and nothing bad will happen to you for doing it.
- You have the right to stay connected to your family, culture, and community.

Expectations

Keep it brief and respectful: 'We want this to feel like home. We just need everyone — including staff — to treat each other with respect. Here's what that looks like in practical terms...'

Questions

'I know that's a lot of information. Do you have any questions right now? You can always come back to me — or any staff member — any time.'

Orientation completed by: _____ Date: _____

Youth acknowledged orientation (or documented reason if declined):

APPENDIX C — RIGHTS INFORMATION HANDOUT — YOUTH VERSION

Provide at admission. Post in the common area. Available in other languages and formats on request.

YOUR RIGHTS — YOU ARE IN CHARGE OF THESE

- You have the right to be **SAFE** — no one can hurt you, threaten you, or punish you physically
- You have the right to know what is happening in your life and to have a say in decisions about you
- You have the right to your own **SPACE** and belongings — your room is yours; your things are yours
- You have the right to **CONTACT** your family and people who matter to you (within your care plan)
- You have the right to go to **SCHOOL** and to get medical care when you need it
- You have the right to your **CULTURE**, language, and community
- You have the right to have your **GENDER IDENTITY** respected by everyone here
- You have the right to make a **COMPLAINT** — nothing bad will happen to you for speaking up
- You have the right to contact **OUTSIDE PEOPLE** privately, at any time, without permission

Who to Call	Number
Emergency — Police / Fire / Ambulance	911
Helpline for Children (24/7)	1-800-663-9122
Advocate for Children and Youth (free, confidential)	1-800-476-3933
BC Ombudsperson	1-800-567-3247
Kids Help Phone (24/7)	1-800-668-6868
Crisis Line BC (24/7)	1-800-784-2433
Your Social Worker	Name: _____ Phone: _____
This Program's Manager	Name: _____ Phone: _____

APPENDIX D — CRITICAL INCIDENT REPORT FORM

Complete for every Category 1 and Category 2 Critical Incident. Verbal notification to MCFD social worker precedes this form. File in youth's ISR. Retain permanently.

Field	
Youth Name (Legal)	
Preferred Name / Pronouns	
Date and Time of Incident	
Location of Incident	
Report Completed By (name and title)	
Date and Time Report Completed	
Supervisor Notified — Name and Time	
MCFD Social Worker Notified — Name and Time	
Method of MCFD Notification	☐ Phone call ☐ PCS after-hours (1-800-663-9122) ☐ Email (non-urgent)
911 Called?	☐ Yes — Police ☐ Yes — Ambulance ☐ Yes — Fire ☐ No
Other Youth Present?	☐ Yes (document separately as required) ☐ No

Incident Category	☐ Category 1 — Same-day verbal + written within 24 hrs ☐ Category 2 — Written by next business day
Incident Type	☐ Injury/medical ☐ Self-harm ☐ Suicidal crisis/attempt ☐ Missing youth ☐ Physical restraint used ☐ Abuse/neglect allegation ☐ Police involvement ☐ Property damage ☐ School suspension ☐ Medication error ☐ Other: _____

Report Section	Details (factual and objective — describe what was observed, not interpretations)
1. Chronological description of the incident	
2. Youth's own account of the event (their words)	
3. Immediate actions taken by staff	

4. Youth's physical and emotional status following the incident	
5. Contributing or contextual factors	
6. Corrective actions required (with owner and target date)	
7. Was an ISP/BSP review triggered? Y / N Scheduled for:	
8. Were other youth affected? Y / N If yes, how addressed:	

Completing Staff Signature / Date	Program Manager Signature / Date	MCFD SW Acknowledgment / Date

APPENDIX E — BEHAVIOUR SUPPORT PLAN TEMPLATE

Develop collaboratively with the youth. Review at every ISP review. Update within 5 days of any significant behaviour incident.

Field	
Youth Name	
Date of Birth	
BSP Author	
Date Developed	
Review Date	
MCFD Social Worker	

Section 1 — Understanding This Youth

Element	Detail (developed with the youth's input wherever possible)
What this youth is good at; their strengths and interests	
What matters most to this youth (people, activities, identity, goals)	
Known triggers — what tends to lead to escalation for this youth	
Early warning signs specific to this youth	
What the youth says helps them when they are struggling	
What does NOT help (and may make things worse)	
Key people who ground and regulate this youth	
Cultural, spiritual, or identity factors relevant to this youth's care	

Section 2 — De-escalation Strategies (in sequence)

Stage	Staff Response for THIS Youth Specifically
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Early signs of dysregulation	
Active escalation	
Crisis (potential harm to self or others)	
Post-crisis recovery (what does this youth need to re-regulate?)	

Section 3 — Replacement Behaviours and Skill Development

Target Behaviour to Replace	Replacement Behaviour / Skill to Build	How This Is Being Taught and Supported

Section 4 — Authorized Interventions

Document only interventions that have been agreed with MCFD and for which staff are trained:

Intervention	Conditions for Use	Staff Authorization Required
Verbal de-escalation (all staff)	Always the first response	All staff
NVCI-authorized supportive hold (if applicable)	Imminent physical harm only; see Section 19	Current NVCI certification required
Other (describe):		

Youth Input Documented / Date	Program Manager Signature / Date	MCFD Social Worker Signature / Date

APPENDIX F — MEDICATION ADMINISTRATION RECORD (MAR)

Complete a separate MAR for each calendar month. Document every administered, refused, or missed dose immediately. Retain for minimum 10 years.

Youth Name	
Date of Birth	
BC Personal Health Number	
Allergies (ALERT — document all)	
MAR Month / Year	
Prescribing Physician(s)	
Dispensing Pharmacy	

Administration Key: Write initials + time in each cell. For refusals: R + time. For missed doses: M + reason. All exceptions require an incident note in the shift log.

Medication / Dose / Route	Time(s)	Days 1-15	Days 16-31

Initials Key (all staff who administered medications this month sign below):

Initials	Full Name (Printed)	Signature

Program Manager Reviewed (monthly) — Signature / Date	
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APPENDIX G — SAFE TRANSITION PLANNING CHECKLIST

Initiate at minimum 6 months before planned transition. Review monthly until discharge. File in ISR.

Field	
Youth Name	
Current Placement Date	
Planned Transition Date	
Transition Destination	
MCFD Social Worker	
Transition Plan Meeting Date	

Transition Task	Done?	Notes
Youth has been meaningfully involved in transition planning	☐	
Youth's goals and concerns about transition documented	☐	
Youth knows where they are going, when, and why	☐	
Transition Plan documented in ISP	☐	
Receiving caregiver/program has received: full ISP	☐	
Receiving caregiver/program has received: health care file and medication list	☐	
Receiving caregiver/program has received: BSP	☐	
Receiving caregiver/program has received:	☐	

Cultural Plan		
School transfer arranged; new school has records and IEP	☐	
New family physician identified and appointment made	☐	
Medication supply (minimum 2-week supply) provided	☐	
All personal belongings inventoried and packed	☐	
Youth has personal identification documents	☐	
Cultural connections confirmed in new setting	☐	
Community supports confirmed: mental health, recreation, employment	☐	
For youth aging out: AWA explored; housing confirmed; income supports	☐	
Farewell activity planned per youth's preference	☐	
Discharge Summary completed within 5 business days	☐	

Program Manager / Date	MCFD Social Worker / Date	Youth (if willing) / Date

APPENDIX H — CULTURAL PLAN TEMPLATE FOR INDIGENOUS YOUTH

Develop within 30 days of placement with the youth, their family, and their Nation. Update at every ISP review.

Field	
Youth Name	
Nation / Band / Community	
Geographic Community / Territory	
Delegated Aboriginal Agency (if applicable)	
Cultural Support Worker / Liaison	
Cultural Plan Developed By	
Date	
Review Date	

Cultural Plan Element	Detail (developed with the youth's active input)
Youth's current relationship with their cultural identity and community	
Youth's own goals for cultural connection during this placement	
Indigenous Nation, language group, and any band affiliation	
Family members and community people who support the youth's cultural connection	
Elders, Knowledge Keepers, and cultural mentors to be involved	
Specific cultural activities, ceremonies, and practices to be facilitated	
Frequency and format of cultural activities planned	

Traditional foods to be incorporated into meal planning	
Traditional medicines or healing practices to be supported	
Language learning: Is the youth interested? Resources identified:	
Friendship Centre or urban Indigenous organization connections	
DAA contact and involvement frequency	
How Evergreen Community Care will support this plan operationally	
How MCFD will support this plan	
How the Nation/community will be involved	
How the youth and their Nation will know this plan is working	

Youth (if willing) / Date	Family / Community Rep / Date	MCFD Social Worker / Date	Program Manager / Date

APPENDIX I — YOUTH COMPLAINT AND FEEDBACK FORM

You can use this form to tell us if something is wrong or to give feedback. You can also talk to any staff member or contact your social worker, the Advocate (1-800-476-3933), or the Ombudsperson at any time. You will not get in trouble for using this form.

Your Name (you can stay anonymous if you prefer)	
Date	
Your concern is about:	☐ A staff member ☐ Another youth ☐ The program or rules ☐ Something in the home ☐ Other: _____

What happened? (Tell us in your own words — as much or as little as you want to share):

What would you like to happen?

Did you feel safe raising this concern? ☐ Yes ☐ No ☐ Not sure

Can we follow up with you? ☐ Yes ☐ I prefer to stay anonymous

For Staff Use — complete same day received	
Received by (name and date)	
Acknowledged to youth on / method	
Program Manager notified on	
Investigation completed by / date	
Response provided to youth on / method	
Complaint resolved? Y / N Notes:	
Escalated to Executive Director? Y / N	

APPENDIX J — MONTHLY PROGRAM QUALITY REVIEW TOOL

Completed by Program Manager. File with the QA Program Binder. Used to track trends and direct continuous improvement.

Facility / Program	
Review Month / Year	
Reviewer (Program Manager)	
Date Completed	

Quality Indicator	Result / Status	Action Required / Owner / Date
Total Critical Incidents this month (Categories 1, 2, 3)		
All CI reports submitted to MCFD within required timelines? Y/N		
CI trend (same youth, same type as last month?): Y/N		
Physical restraints this month: number and circumstances		
All ISPs reviewed on schedule this month? Y/N		
BSPs updated following any incident? Y/N		
Medication count discrepancies this month: number		
Medication errors this month: number and type		
Shift log quality (sample review of 3 logs per youth): rating 1-5		
Training compliance: all staff current?		

Y/N; gaps identified:		
Individual supervision completed for all staff? Y/N		
Team meeting held? Y/N Date:		
School attendance summary: % of scheduled days attended		
Number of youth who had family contact this month		
Number of community outings completed per ISP plans		
Youth complaints received: number and category		
Complaints resolved within required timeline? Y/N		
Facility safety inspection completed? Y/N Deficiencies:		
Staffing below minimum on any shift? Y/N Circumstances:		
Open corrective actions from last month: status of each		
Cultural Plan activities completed per plan? Y/N (for Indigenous youth)		

Program Manager Summary — Key Findings and Priorities for Next Month:

Program Manager Signature: _____ Date: _____

APPENDIX K — TRANSPORT AND COMMUNITY OUTING SAFETY PLAN

Complete before any off-site activity involving youth. File in shift log. Retain for 1 year.

Activity / Destination	
Date	
Departure Time	
Expected Return Time	
Staff Lead	
Additional Staff	
Youth Attending (names)	
Transport Method	

Safety Checklist Item	Done?	Notes
Each youth's ISP reviewed for activity restrictions or supervision requirements	☐	
Staff-to-youth ratio meets ISP requirements for each youth attending	☐	
Risk level for this activity and group assessed: Low / Medium / High	☐	
Emergency medications identified and brought (EpiPen, puffer, other)	☐	
Emergency contacts for all youth in staff bag or phone	☐	
Facility phone number and supervisor on-call number confirmed	☐	

Vehicle inspected before departure; seatbelts and child restraints confirmed	☐	
Destination assessed for hazards (water access, traffic, crowd density)	☐	
Head count completed before departure	☐	
Head count frequency during outing: every _____ minutes	☐	
Head count confirmed on return to facility	☐	
Outing documented in shift log on return	☐	

Incident / Concern during outing (if any):

Staff Lead Signature / Date	
Program Manager Review (if incident) / Date	

APPENDIX L — STAFF TRAINING COMPLIANCE MATRIX

Updated monthly by Program Manager. Staff with expired mandatory training are not assigned duties requiring that training. 'Current' = completion date within renewal window. Highlight gaps in red.

Program / Facility	
Month / Year	
Updated By	

Training Requirement	Staff 1 / Date	Staff 2 / Date	Staff 3 / Date
NVCI (annual)			
Indigenous Cultural Safety (annual)			
Gender-Affirming Care (annual)			
Trauma-Informed Care (2yr)			
Suicide/Self-Harm Response (annual)			
Mandatory Reporting (2yr)			
Medication Admin (annual — if authorized)			
First Aid CPR Level C (annual)			
Workplace Violence Prevention (annual)			
WHMIS 2015 (3yr)			
Working Alone (annual — if applicable)			

Training Requirement	Staff 4 / Date	Staff 5 / Date	Staff 6 / Date
NVCI (annual)			
Indigenous Cultural Safety (annual)			
Gender-Affirming Care (annual)			
Trauma-Informed Care (2yr)			
Suicide/Self-Harm Response (annual)			

Mandatory Reporting (2yr)			
Medication Admin (annual — if authorized)			
First Aid CPR Level C (annual)			
Workplace Violence Prevention (annual)			
WHMIS 2015 (3yr)			
Working Alone (annual — if applicable)			

Enter completion date (MM/YY) or expiry date in each cell. Highlight any cell where training has expired or is within 30 days of expiry.

Reviewed and signed by Program Manager: _____ Date: _____